Housing First, Children Later[1]

Shahar Rotberg[2]

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1 Introduction

On a single night in January 2024 the Department of Housing and Urban Development's point in time count found 771,480 people homeless in the United States, the largest number since the count began and eighteen percent above the year before. In July 2025 the federal government moved decisively to one side of a long argument about what to do about it. An executive order redirected federal homelessness funding away from Housing First, instituted sobriety requirements in federally funded housing and homelessness programmes, and encouraged the civil commitment of people judged unable to care for themselves; the 2026 grant competition cut permanent supportive housing from about three billion dollars to 2.4 billion and moved 1.3 billion into transitional programmes, against about 218 million the year before. Whether housing assistance should be conditioned on sobriety is no longer an academic question.

The argument is usually framed as a contest between two behavioural claims. Critics of Housing First say that unconditional assistance is moral hazard: guaranteeing housing whatever a household does weakens the incentive to stay sober and attached to the labour market, so use and disorder rise. Critics of Treatment First say that conditionality delays stable housing for exactly the households least able to wait, worsens their employment, and makes recovery harder, since recovery is harder on the street. Both claims are true about behaviour, and the experimental literature has not settled between them. Randomised trials of Housing First find large and persistent gains in housing stability and mixed to null effects on substance use (Tsemberis, Gulcur and Nakae, 2004; Padgett et al., 2006; Larimer et al., 2009; Goering et al., 2014; Aubry et al., 2016), but they run six to twenty four months and cannot speak to long run behaviour, to housing markets, or to the budget.

This paper starts from the observation that the debate as framed cannot be settled, because the moral hazard both sides argue about is not an inefficiency. If households are forward looking and face the true hazard of acquiring a substance use disorder, which is what the evidence supports (Lundborg and Lindgren, 2002; Greenwood, Guner and Kopecky, 2025), then at the same generosity and the same cost Housing First weakly dominates Treatment First for every household. The two policies differ only in whether the voucher is withheld in one state, so the budget set under Housing First is weakly larger everywhere and strictly larger somewhere, and a household under Housing First can reproduce its Treatment First outcome exactly by declining to use. Conditionality removes an option from an optimiser and gives nothing back. The extra use that Housing First induces is real, and this paper measures it at +1.26 percentage points, but it is a household optimally taking a risk it is now better able to bear rather than a wedge for policy to close. It is not, on its own, an argument for conditionality.

The case for Treatment First therefore has to rest on something outside the household's own objective, and this paper insists on keeping three such things apart, because the debate does not. The first is a genuine externality: a cost the household imposes on someone whose welfare it does not fully count. The second is the distortion from the taxes that pay for the programme, since a policy that costs more to run raises the rate everybody faces. The third is the programme's own design, since rationing a voucher by lottery is not the same instrument as rationing it by conduct. Only the first is an externality. A conclusion that leans on the second or the third is a different claim about a different object, and conflating them is what allows the same evidence to be read either way.

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[2]Canada Mortgage and Housing Corporation (CMHC). Contact: `shahar.rotberg@gmail.com`. Website: `www.shaharrotberg.com`. This work was conducted on personal equipment and time. The code used to solve the model is the author's own. The opinions expressed herein do not necessarily represent the views of CMHC.

The externality that decides the comparison here is intergenerational. A household that acquires a disorder, or that ends up on the street, passes a worse productivity draw to the child who replaces it, and it counts only a fraction ω of that loss. Each link in that chain is measured. Parental alcohol or drug abuse is recorded as a circumstance at removal for 38 percent of children entering out of home care in 2025, against 18.5 percent in 2000, and in FY2017 that was 96,400 children, or 36 percent of all removals (AFCARS); children of a parent with a disorder are themselves two to four times as likely to acquire one (McGovern et al., 2023). Parental shocks transmit to the child's own earnings: children whose fathers were displaced by a firm closure earn about 9 percent less as adults (Oreopoulos et al., 2008). And housing is one of the environments that does the transmitting: children displaced by Chicago's public housing demolitions earned 16 percent more a year as adults (Chyn, 2018), and a move to a lower poverty neighbourhood before age thirteen raises adult earnings by 31 percent (Chetty et al., 2016). What is a private problem for the parent is a childhood for the child, and the child does not choose it.

A second externality runs through the aggregate rather than through the family: households dislike homelessness around them whether or not it is their own child's, which is what makes the homelessness rate a genuine fixed point problem rather than accounting, and both policies move it (Loubière et al., 2020). It turns out to be much the smaller of the two, and saying so is part of the point: once the channels are separated, one of them carries the result.

The paper makes the argument in two models. A stylized two period model, in Section 3, is stripped to the point where the dominance result above can be proved rather than computed, and where the externality that overturns it can be switched on alone with everything else held fixed. A quantitative general equilibrium life cycle model, in Section 4, then puts the same mechanism in an environment that can be taken to the data: six state variables and fifty three parameters, with endogenous homelessness, drug use, disorder, recovery, serious mental illness, employment, mortality, a drug market that clears, and a government budget that balances. Both models are calibrated to United States data, and the calibration is set out parameter by parameter in Section 5.

One feature of the environment is worth flagging early because it decides who the two policies treat differently. In the data, drug use is concentrated at the bottom of the income distribution. Drugs here are a luxury good bought at a price, so the share of income spent on them rises with income and, left alone, the environment would concentrate use at the top. Mortality risk reverses the gradient: acquiring a disorder truncates the expected continuation value of life, and that loss rises faster than income does, so in equilibrium use concentrates at the bottom without any assumed heterogeneity in tastes conditional on income. The households whose behaviour separates the two policies are therefore the poor ones, which is also where the child externality is largest.

The stylized model delivers the result in a form that can be read off. At universal cover and with no externality, 2.7 percent of households are strictly worse off under Treatment First and none is better off, so the private cost of conditionality is 0.013 percent of consumption in the aggregate: small, because so few households are affected, and unambiguous in sign. Switching the externality on, conditionality prevents 0.217 percentage points of disorder cases, and each prevented case spares one child 54.3 percent of consumption. The product is 0.118 percent, which is 8.9 times the private loss. Housing First is better for every household taken one at a time, and worse once one counts the children of the households it fails to deter. The ranking survives every value of the transmission penalty considered except zero, where by construction there is no externality to price.

The quantitative model is what settles whether that ranking survives in an environment that fits the data, where the drug price responds to the number of users, the rent responds to voucher demand, the tax rate responds to the programme's cost, and the two policies can be separated on generosity as well as on conditionality. It is also what allows the short run and the long run to be distinguished, which matters for reading the experimental evidence: if housing outcomes respond immediately to voucher design while disorder adjusts slowly, short trials are well powered to detect the first and not the second whatever the long run effect turns out to be.

2 Related literature

Five literatures meet in this paper. Four of them supply the machinery and are the ones the previous version of this project drew on; the fifth supplies the motivation and is the one the argument above turns on.

Randomised trials of Housing First. A large empirical literature evaluates programmes that provide rapid housing without conditioning on sobriety, and finds large and persistent improvements in housing stability and reductions in shelter and other public service use (Tsemberis et al., 2004; Padgett, Gulcur and Tsemberis, 2006; Larimer et al., 2009; Goering et al., 2014; Aubry et al., 2016). Evidence on substance use is mixed: most studies find no statistically significant change, and some report modest reductions in alcohol related outcomes. These evaluations typically measure outcomes over six to twenty four months and rarely beyond three years, which is short against a recovery process that Vaillant (1995) documents as gradual and protracted. The model here offers a structural reading of that pattern rather than a dismissal of it: housing responds immediately to voucher design and disorder adjusts slowly, so a short trial is well powered for the first and not the second whatever the long run effect is. The deeper limitation is not power but scope. A trial randomises households within a fixed environment, and holds fixed exactly the things that decide the welfare comparison: the price of the drug, the rent, the tax rate, and the outcomes of children who are not in the sample.

Economic models of drug use. Modelling use as forward looking optimisation dates to Becker and Murphy (1988), and the assumption is empirically defensible at the margin where policy operates. Both users and non users hold risk perceptions close to the objective probabilities (Lundborg and Lindgren, 2002), and Greenwood et al. (2025), who allow perceived and true addiction risk to differ, find the model implied beliefs close to the truth. The price evidence points the same way and is taken up with the drug market below, since the elasticity of participation is what disciplines the size of every behavioural response in this paper. A separate literature emphasises temptation and dynamic inconsistency (Laibson, 1997; Gruber and Kőszegi, 2001), under which present biased households over consume relative to their own long run preferences and corrective policy has a rationale with no externality at all. That literature is complementary here rather than opposed, and it cuts in the same direction: a present biased household would welcome a sobriety requirement as a commitment device, so introducing it would strengthen the case for conditionality rather than weaken it. The dominance result of Section 3.6.1 is therefore the hardest case for Treatment First, not the easiest, which is why the paper is written around it.

Structural models of homelessness. The paper builds directly on İmrohoroğlu and Zhao (2022), who model homelessness as a rational response to income risk, mental health shocks, housing costs and housing policy. In that framework housing policy reaches homelessness through income and housing costs alone, so a sobriety contingent policy has no behavioural margin to act on. Adding endogenous use, disorder and recovery, together with state dependent mortality, creates the margin: the same voucher that insures a household also changes what using costs it, and the disorder it may acquire then feeds back into employment, housing and survival. Abramson (forthcoming) makes the complementary point on the other side of the market, showing that eviction policy changes landlord screening and rents and so changes who becomes homeless in equilibrium. Here the causation runs the other way as well: employment and disorder cost a household its housing, and losing it worsens employment, raises mortality and slows recovery.

What a childhood transmits. This is the literature the paper's motivation now rests on, and it is the one the earlier version of this project did not use. Three links have to hold for the argument to work, and each is estimated separately. That a parent's disorder changes the childhood is documented at scale in the administrative record of the child welfare system, where parental substance abuse is now recorded at more than a third of removals, and in the epidemiology, where household substance abuse is one of the adverse childhood experiences of Felitti et al. (1998) and children of a parent with a disorder are two to four times as likely to acquire one (McGovern et al., 2023). Whether removal itself helps or harms is contested, and nothing here needs it settled: Doyle (2007) finds children on the margin of placement do better left at

home, while Gross and Baron (2022) find placement improved safety and schooling. That a parental shock transmits to the child's own earnings is estimated well outside the substance literature, by Oreopoulos et al. (2008) on paternal job displacement and by Nilsson (2017) on an alcohol availability experiment. And that housing is one of the environments doing the transmitting is the contribution of Chyn (2018) and Chetty, Hendren and Katz (2016), with Collinson et al. (2024) supplying the adult leg from eviction to homelessness. What this paper adds is not another estimate but a price: it puts these magnitudes inside an equilibrium model where the parent's use decision is endogenous, so the transmitted loss becomes a quantity that policy moves rather than a coefficient.

What the hazards are built from. The model's transitions are not free parameters dressed as structure; each is anchored on a clinical or epidemiological estimate, and the calibration section names them one at a time. Four groups do the work. On mortality, Hjorthøj et al. (2023) put the excess death rate for a substance use disorder at six to ten times the baseline, Baggett et al. (2013) measure it for the homeless by age, and Olfson et al. (2015) do the same for serious mental illness; against those hazards the value of a life comes from the willingness to pay estimates surveyed in Viscusi and Aldy (2003). On the mental illness chain, Nielsen et al. (2017) identify the effect of a disorder on the onset of mental illness in Danish registers, Gutwinski et al. (2021) meta analyse its prevalence among the homeless, and Bradizza et al. (2006) review relapse under dual diagnosis. On recovery, Vaillant (1995) documents how slow unassisted remission is, and Milby et al. (2005) supply the causal estimate this paper leans on hardest: a randomised trial on cocaine dependent homeless adults in which the housed arm was abstinent 6.60 weeks against 4.48 in the unhoused arm, which is where both the treatment effect on recovery and the penalty homelessness imposes on it come from. On employment and the cost of housing loss, Desmond and Gershenson (2016) estimate what losing housing does to holding a job, and Loubière et al. (2020) price the disamenity of visible homelessness through a contingent valuation, which is what makes the aggregate headcount enter anybody's utility at all. Taking these from outside the model is the point: the parameters this paper argues over are the ones in Section 5, and a reader who disagrees with one of these is disagreeing with the study behind it.

The drug market. Making the drug price endogenous is what lets a policy that changes the number of users change what using costs everybody else. Two elasticities govern it and they do different jobs, so the literatures behind them are worth separating: the demand side fixes how much use responds to a given price, and the supply side fixes how much the price moves when use changes.

The elasticity of participation. This is the demand side, and it is the better served of the two. Gallet (2014) synthesises 462 price elasticity studies, and van Ours (1995), Saffer and Chaloupka (1999) and Caulkins (2001) establish that participation responds to price even for drugs as habit forming as heroin and cocaine, with long run responses consistently larger than short run ones. That ordering is the signature of forward looking behaviour rather than myopia, which is why it belongs with Becker and Murphy (1988) above as well as here, and the calibration is disciplined to reproduce it. It is what fixes how large the behavioural response to either policy can be.

The elasticity of supply. This is the general equilibrium offset, and it runs against Treatment First in particular: curing households takes them out of the market, demand and the price fall, and a cheaper drug draws others in. No direct estimate exists for an illicit drug. Cannabis is the closest market there is, because it remains a Schedule I substance federally, so even a licensed producer works without interstate trade, without ordinary banking and under federal prohibition, which is the set of frictions an illicit supplier faces; cigarettes and gasoline share none of them. Mace et al. (2020) estimate a price elasticity of supply of 1.46 on Washington State administrative data, and that is the number the model uses.

In a nutshell. The trials establish that housing stability responds to policy but cannot price what happens to anyone outside the sample. The rational addiction literature models the use decision but abstracts from housing and from children. The structural homelessness literature delivers a housing market equilibrium but has no drug margin for conditionality to act on. The transmission literature measures what a childhood costs but takes the parent's behaviour as given. This paper's contribution is to put them in one framework and then to insist on a distinction none of them needs on its own: that the moral hazard which dominates

the policy debate is not a reason for conditionality, and that whatever case exists has to be made on the externality, on the tax, or on the design, one at a time and with the three kept apart.

3 A stylized model

3.1 Three questions

In the data, drug use is concentrated among low income households, most of the homeless do not carry a substance use disorder (SUD), and housing vouchers reach only about a quarter of the households eligible for them. The quantitative model is built to match facts of that kind. It has six state variables and fifty four parameters, which is what the welfare and fiscal questions require, but it also makes it hard to see which pieces of the environment do the heavy lifting. This note strips the environment down to help understand the main drivers in the model, but where my three main questions are still well posed.

Households are perfectly forward looking, here and in the quantitative model. They face the true hazard of acquiring a SUD, they know what carrying one would cost them, and they make no mistakes. Nothing below rests on misperception, on myopia, or on a taste that changes once the household has used.

1. **Who uses drugs?** In the data, use is concentrated at the bottom of the income distribution. Drugs here are a *luxury* good bought at a price: the Stone-Geary form in equation (1) below means the share of income spent on them rises with income, so left to itself the environment concentrates use at the *top*. Which feature of the environment reverses the concentration from the top to the bottom?
2. **How does housing assistance shape the drug use decision?** A voucher pulls drug use in two opposite directions. It raises the value of staying clean, because a housed future is worth more than a homeless one and using puts that future at risk: call this the value of life force, and it lowers use. In contrast, it cushions the consequence of acquiring a SUD, because a household that acquires one is housed anyway: call this the moral hazard force, and it raises use. Both are first order, so theory does not sign the effect of housing assistance on drug use, and the question is quantitative.
3. **When is Treatment First better than Housing First?** In a model without externalities, and where the two programmes cost the same to run, never. Housing First always raises welfare when agents are forward looking, as they are here. A SUD creates a genuine tension, since the same voucher that insures a household also makes the risk it insures more attractive, but that tension does not raise welfare, because Treatment First only removes an option that a household can take under Housing First as well. However, if a SUD and homelessness generate externalities, such as affecting the earning potential of the children of parents with a SUD, then Treatment First may be preferred: children of a parent with a SUD are themselves two to four times as likely to develop one (McGovern et al., 2023), and moving a child to a lower poverty neighbourhood before age thirteen raises adult earnings by 31 percent (Chetty et al., 2016). The model's work here is to show why Housing First raises welfare whenever there are no externalities (and no distortions from the programmes or from the taxes that pay for them, both of which the full model takes up), and then show how the externality that overturns it works.

The model is qualitative throughout: the calibration is rough by design, there is no validation exercise, and every component that obscures the mechanisms has been removed and the model refitted without it. What remains are the main drivers of the results. The welfare evaluations below are for illustration only; the full quantitative model is what settles the main question, which is whether Treatment First raises welfare relative to Housing First in a model that fits the data.

3.2 The environment

3.2.1 Households and preferences

There are two periods and a continuum of households. A period stands for thirty years of adult life, so the two of them span ages twenty five to eighty five. Parameters with a natural annual counterpart are

compounded to that length: the discount factor is β per period, the gross interest rate is $R = 1/\beta$ so that $\beta R = 1$, and ϕ_0 is survival over thirty years. Notation follows the quantitative model throughout.

A household is characterised by its productivity y, fixed over its life and lognormally distributed across households, $\log y \sim \mathcal{N}(-\frac{1}{2}\sigma_y^2, \sigma_y^2)$, so mean income is one. Households differ in y, in age $j \in \{1, 2\}$, in employment status $e \in \{0, 1\}$, in voucher receipt $\eta \in \{0, 1\}$ and in SUD status $d \in \{0, 1\}$, all discussed below, and in nothing else.

Flow utility depends on non durable consumption c, housing services h and drug consumption v,

$$u(c, h, v) \;=\; (1 - \gamma - \theta)\log c \;+\; \gamma \log h \;+\; \theta\big[\log(b + v) - \log b\big] \;+\; \aleph. \tag{1}$$

The three weights sum to one, so γ is the housing budget share. Drugs enter only in the first period, since that is the only period in which v is chosen, and housing is chosen each period from a three point grid $\{h_1, h_2, h_3\}$. The pair (θ, b) governs the drug term: the marginal utility of the vth unit is $\theta/(b + v)$, so θ mostly sets how much a user uses and b whether anyone uses at all. The Stone-Geary form keeps the first unit's marginal utility finite, which is what generates non users at all, because under plain $\theta \log v$ every household uses whatever the consequences. Finally $\aleph$ is the flow value of being alive, the utility of surviving into a period over and above what is consumed in it, which gives mortality a cost that does not vanish when consumption is low. Households save in one risk free asset and cannot borrow.

3.2.2 Timing

Period 1. Every household is clean. Employment $e \in \{0, 1\}$ is drawn with the unemployment probability u_0, and a household that is income eligible draws a voucher with probability π_V; these are the same two lotteries it faces again in period 2. It then chooses consumption, housing, drug use $v \geq 0$, which for simplicity is the only period in which it chooses whether to use, and saving, paying market rent p_r, which is normalised to one, on whatever unit it picks. It starts with no assets. Using drugs generates the risk of a SUD, increasing in v,

$$\pi(v) \;=\; 1 - e^{-\lambda v}. \tag{2}$$

Because π is increasing in v, the household internalises that using more is riskier, so the intensive and extensive margins are not separate decisions.

The transition. The SUD $d' \in \{0, 1\}$ is realised with probability $\pi(v)$ for households that used and with probability zero for the rest, and the household survives into period 2 with probability

$$\phi(d') \;=\; \phi_0\,(1 - \kappa d'), \tag{3}$$

so κ is the proportional reduction in survival caused by the SUD. The mortality penalty applies here, and here only.

Period 2. Terminal, after which the household dies with certainty. A household without the SUD is unemployed with probability u_0 and one with the SUD with the higher probability u_1; if unemployed it receives income support. The voucher is drawn again, subject to the policy rule. The household chooses consumption and housing, there is no drug use decision, and assets are run down with nothing bequeathed.

Mortality, and where the SUD is realised. Mortality is a tax on a continuation value, so the SUD can only cost anything if a future remains when it is carried. That does not require a third period. It requires the SUD to be drawn at a *transition* rather than inside the terminal period, and here it is drawn on the way into period 2, with survival over that transition depending on it, so acquiring it puts the whole of period 2 at risk. Realise it inside the last period instead and death costs nothing because nothing follows it, and the mechanism the model rests on falls silent.

3.2.3 Employment, income support and housing

The gap $u_1 - u_0$ is the entire resource cost of the SUD. Income in a period, given permanent productivity y and employment status e, is

$$y^g(y,e) \;=\; \begin{cases} y & \text{if } e=1, \\ \psi \cdot \min\{y, \bar{y}\} & \text{if } e=0, \end{cases} \tag{4}$$

with ψ the replacement rate and $\bar{y}$ a cap on the earnings base. There is no other income risk. The level of ψ is what links the SUD to homelessness: for the poorest households half of income can fall short of the rent on the smallest unit, so an unemployed household there cannot house itself out of current income.

Each period the household chooses

$$h_k \in \{h_1,\ h_2,\ h_3\}, \qquad h_1 < h_2 < h_3, \tag{5}$$

homelessness (h_1), a small unit (h_2) and a standard unit (h_3). Homelessness is free and delivers less housing services than the market does.[3]

A voucher covers part of the rent, is means tested and is rationed. It is drawn independently each period, which is a simplification relative to the quantitative model, where a voucher persists as long as eligibility holds. Writing $\eta \in \{0,1\}$ for whether one is held,

$$\Pr(\eta' = 1 \mid y', d', v) \;=\; \pi_V \cdot \mathbf{1}\{y' \le y_V\} \cdot \mathbf{1}\{\text{policy covers } (d', v)\}, \tag{6}$$

and a household holding one pays the fraction κ_r of market rent, so the rent paid on option k is

$$r_k(\eta) \;=\; p_r\, h_k \big[\kappa_r \eta + (1-\eta)\big] \cdot \mathbf{1}\{k \neq 0\}, \tag{7}$$

which is zero when the household is homeless. Vouchers operate in both periods, and π_V, the share of eligible households actually covered, is the policy dial of Section 3.5.

The two policies differ in exactly one respect, the last indicator in equation (6). **Housing First** covers every eligible household regardless of (d', v). **Treatment First** covers only households that satisfy the sobriety condition, $v = 0$ and $d' = 0$, which is the rule in the quantitative model as well. Since policy coverage in the states that follow drug use is the only difference, the gap between them is the moral hazard and the whole of it.

The condition binds in period 1 as well, and the timing there is worth stating. Every household arrives clean, so the period 1 voucher is drawn before v is chosen and conditionality cannot screen anyone out on entry. The order within the period is therefore: the household chooses v, the policy rule is then applied to the voucher it drew, and it chooses housing knowing what rent it faces. So under Treatment First a household that uses would be subject to market rent immediately and again in period 2.

3.2.4 The household's problem

Let a be assets carried in. Period 2 is terminal, so the household consumes everything and chooses housing,

$$W_2(a,y,e,\eta,d) = \max_k \Big\{ (1-\gamma-\theta)\log c + \gamma \log h_k + \aleph \Big\}, \qquad c = Ra + y^g(y,e) - r_k(\eta), \tag{8}$$

and write $W_2(a,y,d) \;=\; \mathbb{E}\big[W_2(a,y,e,\eta,d) \;\mid\; y, d\big]$ for its expectation over the period 2 employment and voucher draws. In period 1 the household draws (e,η) and then chooses housing, drug use and saving,

$$V_1(y,e,\eta) = \max_{k,\, v\ge 0,\, a'\ge 0} \Big\{ (1-\gamma-\theta)\log c + \gamma \log h_k + \theta[\log(b+v) - \log b] + \aleph + \beta\, \mathbb{E}_{d'} \Big[\phi(d')\, W_2(a', y, d') \Big] \Big\}, \tag{9}$$

with $c = y^g(y,e) - r_k(\eta) - p_v v - a'$ and $d' = 1$ with the use dependent probability of (2). Survival sits inside the expectation over d' because it depends on which d' is realised.

A household uses if the value at its best positive v exceeds the value at $v = 0$. Households differ only in y and in the draw (e,η), which is the same lottery for everyone, so within a draw the decision defines a set of income intervals on which use occurs, and participation at a given income is the probability of drawing a state in which the household uses.

[3] Two rented sizes is the fewest that lets a household economise on housing rather than lose it. With one, homelessness comes out at 14.40 percent against a target of 0.40, at any h_1.

3.3 Calibration

3.3.1 External calibration

Table 1 lists the parameters set outside the model. Preferences and demographics are conventional: the discount factor over a thirty year period is $\beta = 0.860$[4] and the gross interest rate is $R = 1/\beta$, the dispersion of log permanent income is $\sigma_y = 0.75$ from Heathcote, Perri and Violante (2010), and survival over a period is $\phi_0 = 0.91$, from the Social Security Administration (SSA) period life table for twenty five to fifty five. For the SUD, the proportional fall in survival is $\kappa = 0.49$, derived by compounding an annual mortality ratio of 8, the midpoint of the range in Hjorthøj et al. (2023), so a clean household's chance of dying before period 2 is 9 percent and a household that acquires a SUD faces 53 percent, and the two unemployment probabilities are measured directly. $u_0 = 0.049$ is the headline civilian unemployment rate from the Bureau of Labor Statistics (BLS) Current Population Survey, series UNRATE as distributed by Federal Reserve Economic Data (FRED), averaged over 2014 to 2024. And $u_1 = 0.1625$ is from the National Survey on Drug Use and Health (NSDUH) 2024 for respondents aged 26 and over.

Income support replaces half of previous earnings on a base capped at mean income, in line with statutory state rules. The two rental sizes are set such that the standard unit costs 0.41 of mean income and the small unit is a quarter of it. The voucher parameters are institutional: covered tenants pay 30 percent of market rent, eligibility is capped at mean income, and $\pi_V = 0.27$ is the assisted eligible share in the 2023 Picture of Subsidized Households, published by the Department of Housing and Urban Development (HUD). The weight on drugs is set at $\theta = 0.23$.[5]

3.3.2 Internal calibration

Five parameters are set inside the model, in Table 2, each bracketed on the moment it moves most rather than fitted jointly. The weight on housing γ is matched to the 26.8 percent of total expenditure that shelter plus utilities take in the 2024 Consumer Expenditure Survey (CEX), housing over expenditure net of drugs. The flow value of being alive $\aleph$ is matched to a value of a statistical life of 80 times average annual income, about \$9 million against a mean household income of \$113,000, which is the range surveyed by Viscusi and Aldy (2003) once their estimates are put in current dollars. The pull of the first unit b is matched to a drug participation rate of 5.13 percent from NSDUH 2024 for respondents aged 26 and over, housing services on the street h_1 to a homelessness rate of 0.399 percent from the 2023 HUD point in time (PIT) count, converted to households, and the SUD hazard λ to the 33.2 percent of users who acquire a SUD.

3.4 Who uses drugs

This section asks which feature of the model puts drug use at the bottom of the income distribution, and it starts from the force that any answer has to overcome. Drugs are a luxury good bought at a price, so a household with twice the income gives up less utility per dollar spent on them. With no consequences at all for acquiring a SUD that is the only force operating, and drug use concentrates at the top: the average user then earns 2.62 times the population average.

Two features of the environment are candidates to reverse it. The *mortality channel* is that the SUD increases the chance of dying, through κ in equation (3). The *unemployment channel* is that it costs you a job with probability $u_1 - u_0$, and if income support cannot cover the rent you downsize, live on less, or lose your home. Figure 1 scales each from nothing to 125 percent of its calibrated value, recalibrating b at every point so participation stays at 5.13 percent. The amount of drug use is therefore identical all along both curves and the only thing moving is which households use.

[4]The annual discount rate here is higher than the one in the quantitative model, and this is deliberate. A period here is thirty years, so every consequence of using drugs arrives thirty years after the decision, whereas in the quantitative model consequences arrive annually and compound. A low annual discount factor would weigh that distant death much less and the mortality channel would be losing to impatience rather than to anything the model is about, so a higher factor corrects for the coarseness of the horizon rather than making a claim about preferences.

[5]A θ large enough to match the eleven percent of income that users spend on drugs makes the drug too valuable to be deterred by a death that arrives thirty years later, and users stop being poor. Setting $\theta = 0.23$ gives a spending share of 6.32 percent and keeps the income gradient, which is the trade this model can make.

Table 1: Externally set parameters

Parameter	Description	Source or rationale	Value
Preferences and demographics			
β	Discount factor over a thirty year period	0.99 annually, compounded	0.860
R	Gross interest rate over a thirty year period	Set to $1/\beta$	1.162
σ_y	Dispersion of log permanent income	Heathcote et al. (2010)	0.75
ϕ_0	Survival over a period, no SUD	SSA period life table, ages 25–55	0.91
θ	Weight on drug use	Set; see the text	0.23
The SUD			
κ	Fall in survival with a SUD, over a thirty year period	Hjorthøj et al. (2023), age 26+	0.49
u_0	Unemployment probability, no SUD	BLS CPS via FRED, 2014 to 2024	0.0485
u_1	Unemployment probability, with a SUD	NSDUH 2024, age 26+	0.1625
Income support			
ψ	Replacement rate while unemployed	DOL, statutory UI rates	0.50
$\bar{y}$	Cap on the earnings base, multiple of average income	DOL, state benefit maxima	1.00
Housing, drugs and the voucher			
p_r	Rent per unit of housing services	Normalisation	1.00
p_v	Price of a unit of drugs	Normalisation	1.00
h_3	Standard rental unit	Set	0.41
h_2	Small rental unit	Set	$0.25 \times h_3$
κ_r	Tenant's share of the rent when covered	HUD voucher rules	0.30
y_V	Voucher means test, multiple of average income	HUD income limits	1.00
π_V	Voucher generosity in the baseline	HUD 2023, assisted eligible share	0.27

Note: every parameter here is set from outside, so none has a model counterpart to report against the data. This is the same distinction the Model column of Tables 5 and 6 makes with n.a., which those tables need because they carry externally set and internally calibrated parameters together.

Table 2: Internally calibrated parameters

Parameter	Description	Target moment	Data	Model	Value
γ	Weight on housing services	Housing over non drug spending	26.80%	26.58%	0.289
$\aleph$	Flow value of being alive	Value of a statistical life	80.0	80.0	2.790
b	Pull of the first unit of drugs	Drug participation rate	5.13%	5.13%	0.00316
h_1	Housing services when homeless	Homelessness rate	0.40%	0.42%	0.00573
λ	Addiction hazard per unit of use	Share of users who acquire a SUD	33.2%	35.5%	132.6

The result is sharper than the statement that mortality matters more. That channel takes the income ratio down to 0.45 at full strength, and it does so as a threshold rather than gradually: at half of its calibrated value the ratio is still high, at 3.33, and the reversal happens over the last tenth or so of the channel. Past the calibrated point it keeps biting, reaching 0.19 at 125 percent, so the reversal is not a knife edge that the calibration happens to sit on. The unemployment channel does not reverse it at all, leaving users at 3.33 times average income even fully switched on, and at 125 percent it is 3.33, still above one and no closer to turning. Strengthening the resource penalty by a quarter does not begin to do what mortality does. A resource penalty does not produce the concentration of drug use at the bottom.

A mortality penalty is different in kind. A resource penalty scales with the very thing it is meant to offset, since taking a fixed share of a large income still leaves a large income, so it cannot outrun the fact that drugs are cheaper in utility terms for the rich. What mortality destroys is not resources but a continuation value, and the continuation value rises with permanent income. The household with more to lose loses more, and loses proportionally more than it gains from drugs being relatively cheap. That is the work of $\aleph$ in equation (1): it is the part of the continuation value that does not shrink as consumption falls.

3.5 How does housing assistance shape the drug use decision?

The two forces of question 2 can be separated exactly, because they differ in what happens to a household that uses. Under Treatment First the voucher is withdrawn from any household that violates the sobriety

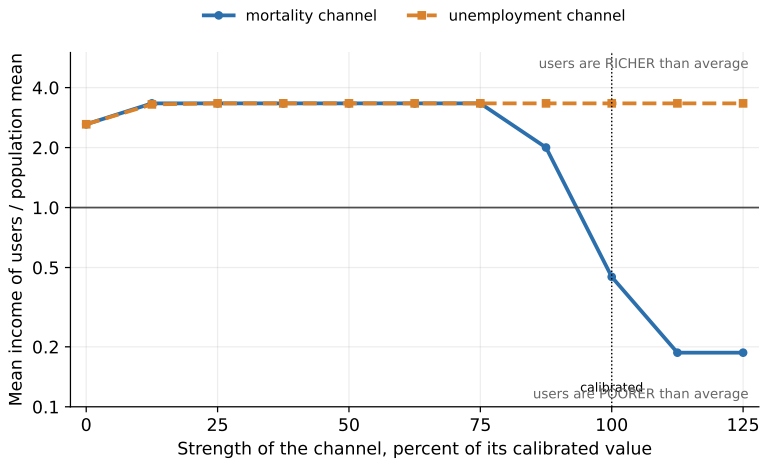


Figure 1: What it takes to reverse the income gradient in drug use

Note: the vertical axis is average income of users over the population average, on a log scale, and b is recalibrated at every point so that participation is 5.13 percent throughout. The black dotted line marks the calibrated strength; everything to the right of it is extrapolation.

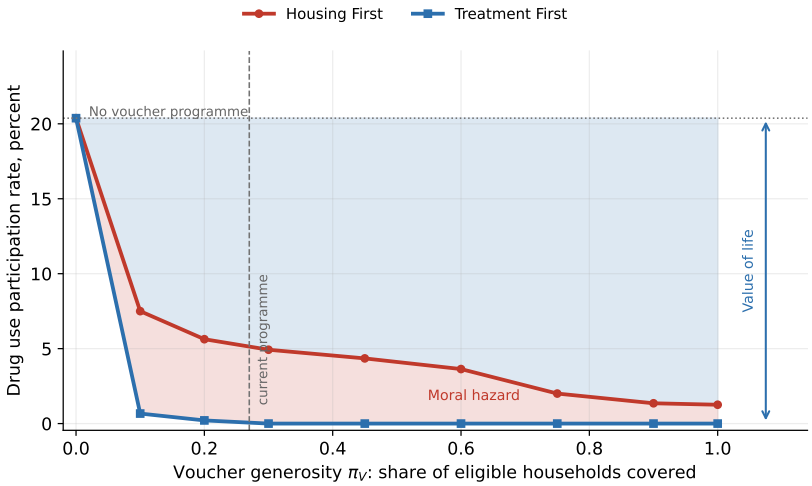


Figure 2: Drug use under the two policies as the voucher becomes more generous

Note: the red band is the moral hazard force and the blue band above it is what Housing First delivers net; together they are the value of life force, marked at the right. The dashed line is the generosity of the current programme, $\pi_V = 0.27$.

condition, so only the value of life force operates. Under Housing First it is not, so both channels operate. Comparing the two policies at the same generosity therefore isolates the moral hazard force, and comparing Treatment First with no programme at all isolates the value of life force.

Figure 2 traces both as generosity rises. With no programme at all, 20.38 percent of households use. Universal Treatment First takes that to 0.00 percent, a value of life force of -20.38 percentage points; universal Housing First leaves it at 1.26 percent, so the moral hazard force is +1.26 points and the net effect of Housing First is -19.12. Both forces are first order and they work against each other, which is the central feature of the problem. Anyone asserting that housing the homeless must increase drug use, or must reduce it, is making a quantitative claim that needs to be tested.

Figure 3 plots the homelessness and SUD outcomes for each programme as its generosity changes, along-

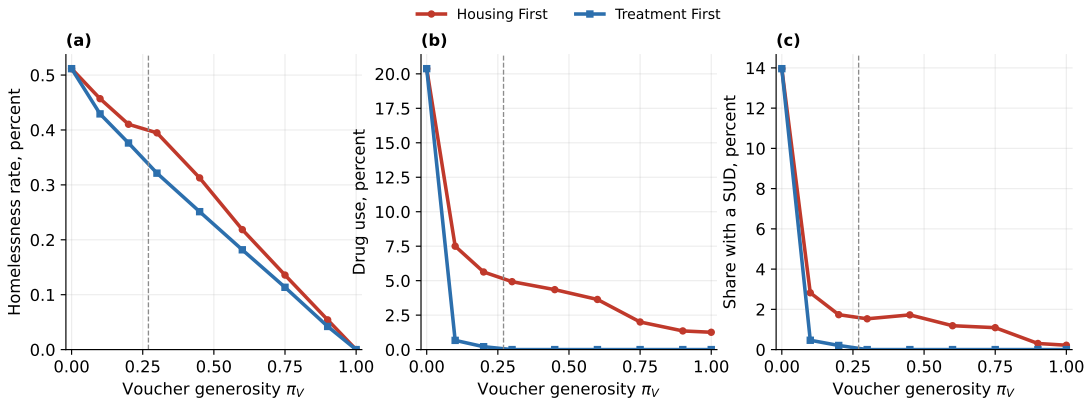


Figure 3: Homelessness, drug use and the SUD under the two policies

side drug use. In panel (a) the two policies end in the same place on homelessness, both driving it from 0.51 percent with no programme to zero, and along the way Treatment First is if anything slightly below Housing First, because the households it deters from using never acquire the SUD that puts their housing at risk. Panel (b) repeats Figure 2 here for convenience. The larger difference is in panel (c), where at universal cover Housing First leaves 0.217 percent of the population carrying the SUD and Treatment First leaves 0.000 percent. What the moral hazard buys is extra SUD cases among households that are housed either way.

3.6 Welfare

3.6.1 Without externalities, Housing First always wins

Sections 3.4 and 3.5 establish what each policy does to behaviour. This section asks what each does to welfare, comparing the two at the same generosity π_V, whose outcomes are in Panel A of Table 3. The policies differ only in the last indicator in equation (6), so Housing First transfers resources in a superset of the states in which Treatment First does. The budget set is weakly larger in every state and strictly larger in one, so a household that optimises cannot be worse off under Housing First:

$$V_1(y;\ \mathrm{HF},\ \pi_V)\ \geq\ V_1(y;\ \mathrm{TF},\ \pi_V) \qquad \text{for every } y, \tag{10}$$

with equality exactly for households that never use. Put the other way round, a household under Housing First can reproduce its Treatment First outcome exactly by not using: conditionality removes an option from a forward looking optimiser and gives nothing back. This is a dominance argument, not a calibrated result, and no choice of parameters overturns it.[6]

It follows that the moral hazard is not an inefficiency. Households here face the true SUD hazard and make no mistakes, so the extra use Housing First induces is not a wedge to be closed but a household optimally taking a risk it is now better able to bear. The +1.26 points of Section 3.5 are a statement about drug use and not about welfare. Panel B of Table 3 puts numbers on it: 2.7 percent of households are strictly worse off under Treatment First, none is better off, and those affected lose 0.50 percent of consumption. Those are exactly the households that use: a household that never uses faces the same voucher rule under both policies and is indifferent between them. The figure is not the 5.13 percent participation rate of Section 3.3 because the programmes compared here are universal, rather than the rationed benchmark that rate is measured at.

A fixed budget does not rescue the welfare case for conditionality either. Treatment First is almost exactly as expensive as Housing First at universal cover, 0.3185 against 0.3184, so matching on cost rather

[6]It does, however, take the price of the drug and the cost of the programme as given, both of which this model fixes and the quantitative model does not. There the tax rate that pays for the programme is endogenous, so a policy that costs more raises the tax on everybody, and the drug price moves with the number of users. Either can separate the two policies with no externality present at all, which is why the argument here is stated for this model and not asserted of the other one.

Table 3: Welfare at universal cover, with no externality

	Housing First	Treatment First
Panel A. Outcomes at universal cover		
Drug participation rate	1.26%	0.00%
Share with a SUD	0.196%	0.000%
Homelessness rate	0.000%	0.000%
Voucher outlay per household	0.3185	0.3185
Panel B. Treatment First against Housing First, no externality		
Households strictly worse off		2.7%
Households strictly better off		0.0%
Consumption equivalent loss, those affected		0.45%
Consumption equivalent loss, whole population		0.012%

Note: the consumption equivalent is the proportional change in consumption, in every period and state, worth as much as the difference in lifetime value. Voucher outlay is expected spending on a household over both periods, the second discounted and weighted by survival, per unit of average income.

Table 4: The externality on children

Panel A. Transmission: what each income penalty implies			
δ_d	Child's SUD rate	Ratio	Loss to the child
0.00	2.24%	1.00	0.0%
0.20	4.04%	1.80	38.7%
0.30 (calibrated)	5.45%	2.43	69.2%
0.40	7.43%	3.31	113.3%
Panel B. Does the externality reverse the ranking?			
Private welfare loss from conditionality			0.012%
Disorder cases prevented			0.196 pp
Damage to the child of one such case			54%
Externality gain from conditionality			0.106%
Gain over loss			8.9

Note: Panel A is at the baseline programme, $\pi_V = 0.27$, since the transmission ratio in the data comes from an economy with a rationed programme; Panel B is at universal cover, one child per household, no discounting across generations. Losses are consumption equivalents.

than on generosity changes nothing. Two things offset. Survival into period 2 falls from 0.91 to 0.47 with a SUD, so the extra households Housing First puts into the SUD state die sooner and fewer are alive in period 2 to collect a voucher; against that, Treatment First withdraws the voucher from anyone who used, so it pays for fewer tenants while they live. The expected outlay remains the same. So a case for conditionality has to come from an externality, and, in this model, from nothing else: with the two outlays equal and financing outside the model, there is no distortion left to tell them apart.

3.6.2 An externality on children reverses it

I model an externality that runs through income rather than through preferences. I have considered two alternatives, but both seem to be at odds with reality. Letting the child of a user inherit a stronger taste, a lower b, matches the transmission fact but has the wrong welfare sign: b enters only through $\theta \log(1 + v/b)$, so a lower one raises the child's objective at every $v > 0$ and its lifetime value with it. So the specification says that a child who inherits a stronger taste for drugs is *made better off* by inheriting it, and that is the opposite of what these children report. Growing up with household substance abuse is one of the adverse childhood experiences in Felitti et al. (1998), and it predicts higher rates of depression, worse self reported health and lower life satisfaction in adulthood; the children of parents with a SUD also have lower educational attainment, which is the premise of the transmission literature δ_d is calibrated against (McGovern et al., 2023). A model in which the transmitted trait raises the child's own welfare cannot be describing any of that, whatever it does to the transmission rate. Letting it inherit a greater susceptibility, a higher λ, fails

another way, since a child facing a riskier drug optimally uses *less* ex ante, which is at odds with the data: children of parents with a SUD are *more* likely to use, not less.

Income works on both counts, and it works through the channels explored in Section 3.4: lower permanent income is unambiguously bad to receive, and this model already puts users at the bottom, so a child born poorer is mechanically more likely to use, which is aligned with the data. It is also supported by the evidence, and it stands in for more than income alone: a parent who diverts spending and time to drugs invests less in the child's education and development, and the income penalty is the reduced form of all of that (Chetty, Hendren and Katz, 2016; McGovern et al., 2023). The income gradient does the whole job, so the model transmits a SUD across generations *without transmitting any preference for drugs.*

A child's baseline income is drawn from the same lognormal as everyone else, and the entire link is the penalty

$$y_{\text{child}} \;=\; y_{\text{base}}\,(1-\delta_d)^{\mathbf{1}\{\text{parent acquired the SUD}\}}\,(1-\delta_h)^{\mathbf{1}\{\text{parent was homeless}\}}, \tag{11}$$

which the parent does not internalise. That is what makes it an externality rather than something already priced into V_1.[7]

The two penalties are set to different things. Panel A of Table 4 shows that $\delta_d = 0.30$ makes the child of a parent who acquired the SUD 2.4 times as likely to acquire one, in the middle of the epidemiological range of two to four in McGovern et al. (2023) and the alcohol literature. The homelessness penalty δ_h is set to 31 percent, directly from Chetty et al. (2016), who find that children who moved to a low poverty neighbourhood before age 13 went on to earn 31 percent more as adults. The two penalties also do different work: both policies drive homelessness to zero at universal cover, so δ_h prices the programme against no programme while δ_d decides Treatment First against Housing First.

Panel B puts the two sides together and they are not close. The private cost of conditionality, in the first row, is 0.013 percent of consumption, small because only 2.7 percent of households are affected. Against that, conditionality prevents 0.217 percentage points of SUD cases in the second row, and each of those cases costs one child 54.3 percent of consumption, shown in the third row. The gain in the fourth row is the product of those two, since one prevented case spares one child: 0.217 percentage points times 54.3 percent gives 0.118 percent of consumption, which is 8.9 times the private loss of the first row. That ratio exceeds one for every δ_d in Panel A except $\delta_d = 0$, where by construction there is no externality to price. Housing First is better for every household taken one at a time, and worse once one counts the children of the households it fails to deter.

All of this is a crude illustration: the model is two periods long and nobody who faces the sobriety condition fails to meet it, so the impacts of policies can be unreasonably large. It shows that an externality can reverse a welfare ranking that is otherwise airtight, which is all it is for. Which way the comparison actually goes is a question about magnitudes, and that is what the quantitative model of the next section is for.

The quantitative model carries two further channels absent here, and they are different in kind as well as in sign, which is another substantive reason the welfare ranking is a question of magnitudes. The first is not an externality at all but the distortions the programme creates and the distortionary taxes required to pay for it; the second is an externality of the same type as the one on children.

- **Programmes distort the decisions of recipients and are paid for out of distortionary taxes.** A programme lowers the labour supply of the households it does reach, because eligibility is means tested, and the voucher is clawed back at a rate of κ_r, so a recipient who works more loses eligibility or some of the transfer. This, combined with the pure financing of the programmes, means that other households have to pay more taxes, which distorts their labour supply.
- **Homelessness is a disamenity in itself.** Households dislike homelessness around them and not only their own, so the aggregate rate is an argument of everyone's utility. This one is an externality, and it survives any way of paying for the programme. It runs in favour of whichever policy houses more, which is the opposite direction from the tax.

[7] In the quantitative model the altruism of parents towards their children is calibrated rather than set to zero, so that the channel where a parent does take some account of what it does to its child is present and only the remainder is external.

4 The quantitative model

This section describes the model the paper's numbers come from. Where the stylized model already carries something unchanged it is named rather than motivated a second time: the Stone-Geary form of the drug term, the logic that separates the two policies, and the transmission externality all appear below. Notation follows Section 3 wherever the two models share an object: γ and θ are the housing and drug weights in both, b is the same Stone-Geary endowment, $\aleph$ the same flow value of being alive, λ the same SUD hazard, ψ and $\bar{y}$ the same replacement rate and earnings base cap, r the same risk free rate, and β is the discount factor and nothing else.

4.1 Demographics, the state and the choice set

Households enter as adults, live at most J annual periods and retire at J_R, and j indexes age within that span. Survival from j to $j+1$ is $\phi(j, h', d, m, \tau)$ in (24), and it depends on age, on the housing *chosen* this period, on the SUD and on serious mental illness (SMI), and on whether the household is in treatment, since treatment means it is not using, and thus is less likely to die. Newborns replace the dead exactly, so the mass of households is constant. The individual state is

$$s = (a,\, y,\, e,\, d,\, m,\, \eta) \in \mathcal{A} \times \mathcal{Y} \times \{1,2,3\} \times \{0,1\}^3, \tag{12}$$

where a denotes assets, y permanent productivity, e employment status, d the SUD, m SMI and η receipt of a voucher.

Each period the household chooses

$$\big(n,\, h',\, v,\, \tau,\, a'\big) \in [0,1] \times \{h_1, h_2, h_3, h_4\} \times \mathbb{R}_+ \times \{0,1\} \times \mathcal{A}, \tag{13}$$

where n denotes hours, h' the housing unit chosen, of which h_1 is homelessness, v drug use, τ SUD treatment take up, available only when $d = 1$ and requiring $v = 0$ in the period it is taken, and a' next period assets.

4.2 Preferences

Period utility is

$$u\big(c, h', v, n, \tau; d, m\big) \;=\; \frac{\left[\, c^{1-\gamma-\theta}\,(h')^{\gamma}\,(v+b)^{\theta}\,\right]^{1-\sigma}}{1-\sigma} \;+\; \omega_\ell \frac{(1-n)^{1-\sigma_\ell}}{1-\sigma_\ell} \;+\; \aleph \;-\; \tau\bar{\tau}^{u} \;-\; \kappa_x H, \tag{14}$$

Four of these are the stylized model's and carry the same meaning here as in Section 3.2: γ the housing weight, θ the drug weight, so that $1-\gamma-\theta$ is the weight on consumption and the three sum to one; b the Stone-Geary endowment; and $\aleph$ the flow value of being alive. Equation (1) is the logarithmic case of this bundle with hours fixed.

The rest are new, and there are three of them. Leisure makes hours a margin, at weight ω_ℓ and curvature σ_ℓ over the $1-n$ hours not worked. SUD treatment costs utility, $\bar{\tau}^u$ in each period the household takes it up, $\tau = 1$. And homelessness carries a disamenity, κ_x per unit of the aggregate homelessness rate H, borne by every household and priced on the headcount alone, which is what makes homelessness an externality here rather than only a private misfortune; Section 5.1 takes it up. Outside the bundle, σ is the coefficient of relative risk aversion.

4.3 Income, taxes and transfers

Gross income depends on age, employment, productivity and health,

$$y^g(j,y,e,d,m,n) = \begin{cases} 0.40\,y & j \geq J_R \quad \text{(social security)} \\ n\,y\,\varepsilon(j)\,\rho_d^{\mathbf{1}\{d=1\}}\rho_m^{\mathbf{1}\{m=1\}} & j < J_R,\ e=1 \quad \text{(employed)} \\ \psi\ \min\{y\,\varepsilon(j),\ \bar{y}\} & j < J_R,\ e=2 \quad \text{(first year of a spell)} \\ \zeta_{DI}\,y\,\varepsilon(j) & j < J_R,\ e=3,\ m=1 \quad \text{(non employed with SMI: disability)} \\ 0 & \text{otherwise,} \end{cases} \tag{15}$$

taking the cases in the order they appear. Retirement income replaces 40 percent of the productivity state with no health penalty, since social security is not means tested on health. For the employed, $\varepsilon(j)$ is the deterministic age profile of efficiency and ρ_d and ρ_m are productivity penalties applied *conditional on working*. The three employment states are work, $e=1$, the first period of a spell out of work, $e=2$, and non employment thereafter, $e=3$, so the last two cases are both unemployment and the disability case is the income of a household that is out of work and carries SMI. Unemployment insurance (UI) is paid in the first period of a spell only, at the replacement rate ψ on an earnings base capped at average income $\bar{y}$, both as in equation (4). ζ_{DI} is the rate at which disability insurance (DI) replaces the income of a non employed household with SMI.

The two penalties, ρ_d and ρ_m, are in the model to generate a large share of homeless households being employed, consistent with the data. Employed homelessness cannot be produced by unemployment alone: something has to leave a household poor while it is still in a job. Both are calibrated internally: ρ_d on the share of the homeless who are employed, and ρ_m on the earnings of workers with SMI relative to average.

Labour income is taxed with the Heathcote et al. (2017) function and capital income at a flat rate τ_k,

$$y^n = \tau_\ell\,(y^g)^{\xi} \ +\ (1-\tau_k)\,r\,a, \tag{16}$$

with y^n denoting after tax income, where ξ governs progressivity and τ_ℓ the level. Households can receive two transfers, both means tested on gross income as the programmes themselves are. A food stamp floor tops means tested income up to $\underline{c}\,\bar{y}$, with $\underline{c}$ the floor expressed as a share of average income, and the voucher covers rent above a tenant contribution of κ_r of gross earnings, the same κ_r as in Section 3.2.3,

$$F = \max\big\{0,\ \underline{c}\,\bar{y} - (y^g + ra)\big\}, \qquad Vch = \mathbf{1}\{\eta=1\}\,\mathbf{1}\{h' \neq h_1\}\,\max\big\{0,\ p_r\,h' - \kappa_r\,y^g\big\}. \tag{17}$$

A homeless voucher holder does not lose the voucher: retention depends on income eligibility, not on housing, so the entitlement survives and simply pays nothing until the household rents again.

4.4 The budget constraint

$$c + p_v v + a' \ =\ y^n + a \ -\ p_r\,h'\,\mathbf{1}\{h' \neq h_1\} \ +\ Vch + F, \qquad c>0,\ v\geq 0,\ a'\geq 0. \tag{18}$$

The drug price is p_v and market rent is p_r, both as in Section 3.2.3, and a is assets carried in. Note what is absent: treatment, which costs utility, and bequests, which go to the government. How c, v and n are chosen out of it is with the rest of the optimisation, in Section 4.11.

4.5 Housing, and the homelessness loop

Housing is a four point grid. h_1 is literal homelessness, sleeping rough or in an emergency shelter, which is the HUD point-in-time definition and excludes doubling up; it carries a small service flow and no rent, a fixed fraction of the smallest rental rather than a level of its own. The three rental sizes are pinned at both ends and spaced by a curvature between them,

$$h_2 = \underline{h}, \qquad h_3 = \underline{h} + \Big(\tfrac{1}{2}\big(\bar{h}-\underline{h}\big)^{1/\varsigma}\Big)^{\varsigma}, \qquad h_4 = \bar{h}, \tag{19}$$

so three parameters describe the grid: $\underline{h}$ the smallest rental size, $\bar{h}$ the largest, and ς the curvature that places the rung between them. At $\varsigma = 1$ the ladder is uniform and at $\varsigma > 1$ the middle rung sits closer to the bottom, which is what a housing size distribution with most households in the largest category requires.

Homelessness then enters three separate equations: survival (24), the employment transitions (26) and the SUD recovery hazard (21), through $\kappa_{\phi H}(j)$ and κ_{ueH} in Section 4.8 and μ_H in Section 4.6. It raises mortality, worsens both employment margins and lowers the chance of recovering from the SUD. Mental illness is the one thing it does not reach. The SMI chain runs on its own, and that is a finding rather than an omission: the trial that housed people with serious mental illness looked for an effect on their symptoms and did not find one, while finding large effects on their housing. Section 4.7 gives it.

4.6 Drug use, the SUD, recovery and treatment

A household either abstains or uses, with v chosen continuously. A clean household that uses acquires the SUD with the hazard of equation (2),

$$\Pr(d' = 1 \mid d = 0, v) = \pi_d(v) = 1 - e^{-\lambda v}, \tag{20}$$

In the baseline a clean household may initiate only up to an age J_E, the last age at which use can begin.[8] A household with the SUD recovers with probability

$$\pi_r(h', m, \tau) = \min\left\{1,\ \bar{\pi}_r\, \mu_H^{\mathbf{1}\{h'=h_1\}}\, \mu_m^{\mathbf{1}\{m=1\}}\, g^{\tau}\right\}, \tag{21}$$

with $\bar{\pi}_r$ the baseline annual chance of recovering while housed, clean of SMI and untreated, and three multipliers on it: $\mu_H < 1$, so recovery is harder on the street, $\mu_m < 1$, harder with SMI, and $g > 1$ the treatment effect.

μ_H is how housing acts on drug use here, and it is worth being exact about what that claim is and is not. Housing does not change what a household wants: flow utility is the same function of (c, h, v) housed or not. What housing changes is the chance of getting out of the disorder once it is carried, recovery on the street being μ_H times recovery while housed. Milby et al. (2005) randomised cocaine dependent homeless adults into arms differing in housing and measured weeks of abstinence, so the ratio between arms identifies μ_H in our calibration.

Treatment τ is a *choice*, not a mechanical hazard, taken at the utility cost $\bar{\tau}^u$ in (14). It also costs the drug: a household in treatment must abstain in that period, $v = 0$ whenever $\tau = 1$, which is imposed as a constraint on (30) rather than priced. On the other hand, because it is not using, its mortality rate goes back to the baseline for that period.

g is the parameter through which its entire effect runs, so treatment has to be something a household decides to undertake and can decline.

4.7 Serious mental illness

m is an exogenous two state Markov chain representing SMI, whose prevalence $p_m(j)$ averages about 5.5 percent of adults, defined by substantial functional impairment, rather than any mental illness, which is roughly a quarter of adults and includes mild anxiety: a state that halves productivity cannot sensibly cover a quarter of the population.

The onset rate is a function of age. Writing $p_m(j)$ for prevalence at age j and λ_m for the annual chance of leaving the state,

$$\text{onset}_m(j) = \frac{p_m(j{+}1) - p_m(j)(1 - \lambda_m)}{1 - p_m(j)}, \tag{22}$$

which is the accounting identity that reproduces the observed age profile exactly, backed out of the age profile of prevalence in NSDUH 2023 and the recovery hazard. The transitions are then

$$\Pr(m' = 1 \mid m = 0, d) = \text{onset}_m(j) \cdot \iota^{\mathbf{1}\{d=1\}}, \qquad \Pr(m' = 0 \mid m = 1) = \lambda_m, \tag{23}$$

[8] Without it there is little consequence at old ages to using: too little life remains for the disorder to damage, so the model produces too much of it.

where $\iota > 1$ is the multiplier on onset for a household carrying the SUD, which makes a SUD raise the hazard of SMI. That last channel runs in the direction the evidence supports: substance induced psychosis converts to schizophrenia or bipolar disorder at a pooled rate near a quarter (Murrie et al., 2020), and in the Danish registers any SUD predicts conversion to schizophrenia with a hazard ratio of 1.34, opioid use disorder 2.74 (Nielsen et al., 2017).

SMI then affects five objects: productivity conditional on working through ρ_m in (15), both employment margins through κ_{uem} in (26), mortality through $\kappa_{\phi m}$ in (24), the SUD recovery hazard through μ_m in (21), and disability income, for which eligibility is being out of work with SMI and the replacement rate is ζ_{DI}. It does not change the utility function.

The mental health chain does *not* depend on housing, and that is a finding in the data rather than an omission. In the At Home / Chez Soi randomised trial, 1,198 homeless adults with SMI across four cities, assignment to housing with intensive case management produced no detectable effect on symptom severity at any wave, while producing large effects on housing stability, its primary outcome. Making the channel housing dependent would mean adding a mechanism the best available trial looked for and did not find.

4.8 What the SUD, SMI, and homelessness cost

Death risk. Survival is

$$\phi(j, h', d, m, \tau) = 1 - \min\Big\{1,\ (1 - \bar{\phi}(j))\,\kappa_{\phi H}(j)^{\mathbf{1}\{h'=h_1\}}\,\kappa_{\phi d}(j)^{\mathbf{1}\{d=1\}\mathbf{1}\{\tau=0\}}\,\kappa_{\phi m}^{\mathbf{1}\{m=1\}}\Big\}, \tag{24}$$

with $\bar{\phi}(j)$ the baseline survival age profile and $\kappa_{\phi H}(j)$ and $\kappa_{\phi d}(j)$ two age varying multipliers, and $\kappa_{\phi m}$ one that is not, raising the death hazard for homelessness, the SUD and SMI respectively. The SUD multiplier is switched off in a period the household is in treatment, $\tau = 1$, which is why the survival function takes τ as an argument at all. The excess mortality of the disorder is overwhelmingly a mortality of use, poisoning above all, and treatment forbids use in the period it is taken, so a household that is not using does not carry it. I make this assumption because that is what the cause specific evidence shows. The meta-analyses of mortality among people using extramedical opioids report overdose as the largest single contributor to the excess (Larney et al., 2020; Degenhardt et al., 2011), and in CDC WONDER drug poisoning alone accounts for about a fifth of all deaths at ages 25 to 44, which is the moment Section 5.3 reports the model against. $\bar{\phi}(j)$ is the SSA period life table for ages 26 to 85, with the sexes combined by survivor weight, $q = [l^M q^M + l^F q^F]/[l^M + l^F]$.

The homelessness multiplier cannot be read off the data directly, and the reason is worth spelling out. Baggett et al. report a *standardised mortality ratio* (SMR) for homeless adults: deaths among the homeless at a given age divided by deaths in the general population at that age, which they publish by age band and which we write $RR(j)$. It cannot be used as $\kappa_{\phi H}(j)$ because the people it is measured on are not only homeless. A share $s = 0.217$ of them also carry a SUD (Gutwinski et al., 2021), which raises mortality by itself, so the published ratio is a mixture of two effects and attributing all of it to housing would overstate what housing does.

Three things are observed and two are solved for. Observed: $RR(j)$, Baggett's ratio; $\sigma_d(j)$, the shape of the SUD's mortality effect over age; and s, the SUD share of the homeless. Solved for: $\kappa_{\phi d}(j)$ and $\kappa_{\phi H}(j)$, one age profile from each equation. Picking the single number $\bar{\kappa}_{\phi d}$ settles both: the first equation turns it into $\kappa_{\phi d}(j)$ at every age, and the second then reads $\kappa_{\phi H}(j)$ off Baggett's ratio at every age.

Undoing the mixture needs the SUD's own multiplier at each age. That is what $\sigma_d(j)$ supplies: the age profile of drug poisoning deaths from CDC WONDER 2023, normalised to one at the age where it peaks, so $\sigma_d(j)$ is the share of the SUD's peak mortality effect that is present at age j. Writing $\bar{\kappa}_{\phi d}$ for the level of the SUD multiplier,

$$\kappa_{\phi d}(j) = 1 + (\bar{\kappa}_{\phi d} - 1)\,\sigma_d(j), \qquad \kappa_{\phi H}(j) = \max\Big\{1,\ \frac{RR(j)}{(1-s) + s\,\kappa_{\phi d}(j)}\Big\}. \tag{25}$$

The denominator is the average mortality multiplier in Baggett's population: a share $1 - s$ of it carries homelessness alone and a share s carries the SUD as well. Dividing the observed ratio by it leaves the homelessness effect on its own.

$\bar{\kappa}_{\phi d}$ is assigned from the literature on all cause standardised mortality ratios for opioid use disorder, which report values between 6 and 10 (Degenhardt et al., 2011; Hser et al., 2017; Larney et al., 2020), and the calibration section reports the value taken. The drug price participation elasticity is not targeted; it is reported as a validation moment and compared with the range estimated in the literature, -0.3 to -1.1 (Saffer and Chaloupka, 1999; Grossman and Chaloupka, 1998; Olmstead et al., 2015).

$\kappa_{\phi m}$ carries no age argument because the data give it basically none. Olfson et al. (2015) give all cause standardised mortality ratios for schizophrenia of 3.6, 3.4 and 4.0 across ages 20 to 34, 35 to 54 and 55 to 64. Flat, if anything rising.

Unemployment risk. Job finding and separation are

$$\pi^{ue} = \min\left\{1,\ \bar{\pi}^{ue}\,\kappa_{ueH}^{\mathbf{1}\{h'=h_1\}}\kappa_{ued}^{\mathbf{1}\{d=1\}}\kappa_{uem}^{\mathbf{1}\{m=1\}}\right\}, \qquad \pi^{eu} = \min\left\{1,\ \bar{\pi}^{eu}\,\kappa_{ueH}^{-\mathbf{1}\{h'=h_1\}}\kappa_{ued}^{-\mathbf{1}\{d=1\}}\kappa_{uem}^{-\mathbf{1}\{m=1\}}\right\}, \tag{26}$$

with $\bar{\pi}^{ue}$ and $\bar{\pi}^{eu}$ the baseline job finding and separation rates and κ_{ueH}, κ_{ued} and κ_{uem} the multipliers for homelessness, the SUD and SMI, so each multiplier below one makes work simultaneously harder to find and easier to lose.

The child's income risk. A parent with a SUD, or one who ends up on the street, lowers the productivity (y_{child}) its child is born with, through δ_d and δ_h respectively, as shown in (39) in Section 4.14. That is a risk to the household's own objective only to the extent that ω, the weight the parent places on its child's lifetime utility in (30), is positive. Since ω is small (see Section 5 for details), the household weighs a fraction ω of this risk and $1-\omega$ of it falls outside the problem entirely. It is therefore the one risk here that is principally an externality rather than a pure personal cost, which is what makes it a candidate for policy in a way that mortality and unemployment risk, both fully internalised, are not.

4.9 The voucher, and the two policies

A household without a voucher draws one with probability π_V, the same lottery probability as in Section 3.2.3, if it is income eligible, and a household with one keeps it while eligible, so unlike Section 3.2.3 the voucher is persistent rather than redrawn each period. Eligibility is

$$y^g + ra \ < \ \kappa_V\, y^{\text{med}}. \tag{27}$$

with κ_V the eligibility cutoff expressed as a share of median gross income y^{med}. Under Housing First that is the whole rule. Under Treatment First the voucher is additionally lost with probability p_{det} by a household that uses drugs or has a SUD, and with probability p_{fp} by one that did not,

$$\Pr(\eta'=1) = \Pr(\eta'=1)^{HF} \times \begin{cases} 1-p_{\text{fp}} & \text{if } (v=0 \text{ and } d=0) \text{ or } \tau=1, \\ 1-p_{\text{det}} & \text{otherwise.} \end{cases} \tag{28}$$

There are four cases and the rule covers them. A clean household that does not use meets the condition and faces only the false positive rate p_{fp}. A clean household that uses fails it and faces p_{det}. A household carrying the disorder that does not take treatment fails it and faces p_{det} as well. And a household carrying the disorder that does take treatment is abstaining, $v=0$ by the restriction in Section 4.6, so it meets the condition and faces p_{fp}. There is no fifth case: treatment is available only when $d=1$, and it forbids use, so no household is ever both in treatment and using.

This is the one place where the quantitative model changes the *logic* of Section 3.5 rather than its setting. There, conditionality is perfectly enforced and perfectly met, so Treatment First deters everyone and nobody is thrown off the rolls. Here detection is imperfect and so are households: some fail the condition because they cannot get clean, and some fail it because the test is wrong. Those two groups are the empirical case for Housing First, neither exists in the stylized model, and that is why its welfare numbers were illustration.

4.10 The shocks, collected

Conditional on the state and the choices, next period's state is drawn from five independent transitions. Collecting them in one place:

Productivity y. Fixed over the life cycle, so $y' = y$. I do this for two reasons. First, it is what keeps the child's penalty (39) a shift along a single scalar rather than a shift across the nodes of an age varying discretisation, which significantly reduces the computation time to manageable levels. Second, it still captures the key mechanisms, because unemployment and the penalty ρ_d are the income risks that link the SUD to housing, and those are retained in full.

Employment e. A household that loses their job receives insurance with certainty for one year. With π^{ue} and π^{eu} from (26), it gives the following transitions:

$$e = 1 \;\longrightarrow\; \begin{cases} e' = 1 & \text{w.p. } 1 - \pi^{eu} \\ e' = 2 & \text{w.p. } \pi^{eu} \end{cases} \qquad e \in \{2, 3\} \;\longrightarrow\; \begin{cases} e' = 1 & \text{w.p. } \pi^{ue} \\ e' = 3 & \text{w.p. } 1 - \pi^{ue}, \end{cases} \tag{29}$$

State 2 is reachable only from employment and never from itself, so a household is in it for the first year out of work and in state 3, which pays nothing, from the second year on. One year is if anything generous: most states pay for twenty six weeks.

The SUD d. By (20) if $d = 0$ and (21) if $d = 1$, conditioning on the chosen v, h' and τ.

Mental illness m. By (23), conditioning on d.

The voucher η. By (27) and (28), conditioning on next period's eligibility and, under Treatment First, on the chosen v, the status d and the treatment choice τ.

Write $\Gamma\big(s' \mid s,\, n, h', v, \tau\big)$ for the product of these five, which is the object the expectation in (30) is taken against.

4.11 The household's dynamic problem

Putting it together, for $j = 1, \ldots, J$ and with $V(J{+}1, \cdot)$ given below,

$$\begin{aligned} V(j, a, y, e, d, m, \eta) = \max_{\{n,\, h',\, v,\, \tau,\, a'\}} \Bigg\{ & \frac{\big[c^{1-\gamma-\theta}(h')^{\gamma}(v+b)^{\theta}\big]^{1-\sigma}}{1-\sigma} + \omega_{\ell}\frac{(1-n)^{1-\sigma_{\ell}}}{1-\sigma_{\ell}} + \aleph - \tau\bar{\tau}^{u} - \kappa_x H \\ & + \; \beta\,\phi(j, h', d, m, \tau)\int_{\mathcal{S}} V(j{+}1)(s')\,\Gamma\big(\mathrm{d}s' \mid s, n, h', v, \tau\big) \\ & + \; \beta\,\big[1 - \phi(j, h', d, m, \tau)\big]\,\omega\,\Big[\mathbb{E}\,V\big(1, 0, y_{\text{child}}, e', 0, 0, 0\big) \\ & \qquad - \; \mathbb{E}\,V\big(1, 0, y_{\text{base}}, e', 0, 0, 0\big)\Big] \Bigg\} \end{aligned} \tag{30}$$

subject to the budget (18), the income definitions (15) to (17), the choice set of dwellings (13), $\tau = 0$ whenever $d = 0$, $n = 0$ whenever $j \geq J_R$ or $e \neq 1$, $v = 0$ whenever $\tau = 1$, $a' \geq 0$, $c \geq F$ so that consumption never falls below the food stamp floor, and $V(J{+}1, \cdot) = 0$ with $\phi(J) = 0$, so that at the last age the child is the whole continuation. The integral runs over next period's state $s' = (a', y, e', d', m', \eta')$; the second line of (30) is the household's own continuation, and the third is the child's expected lifetime utility, entered at age one with zero assets and with its own employment status e' drawn as any newborn's is.

A household that does not survive from j to $j + 1$ leaves a child, so the continuation is the survival weighted value of its own next period plus the death weighted value of what the household's own state costs

that child, each discounted, with y_{child} as defined in (39). What that buys is a second route by which the household's own decisions, to use and to go unhoused, reach its child. $\phi(j,\cdot)$ itself depends on the dwelling chosen, so the housing decision moves the weight on the two terms directly. The drug does not move it within the period, because ϕ takes the disorder as a state and v only raises the chance of carrying it *next* period. The drug reaches the child through the continuation instead, which is where the first order condition below picks it up.

The third line is a *difference* and not a level, and the distinction is not cosmetic. Written as a level, ω times the child's utility, the term is worth more to a household the likelier it is to die, because it is the death branch that carries it. A SUD raises the death hazard several fold, so a level would pay most to exactly the households the paper is about, and it would do so for reasons that have nothing to do with the child.

Subtracting the value of a child born to a clean, housed parent removes that. The term is zero for a household in the reference state and negative otherwise, so what enters the decision is only the loss the parent's own state imposes on its child, weighted by the chance that child is born at all.

At $\omega = 0$ the damage a household does to its child is entirely external; at $\omega > 0$ the household internalises a share ω of it, and only $1-\omega$ remains external. Because the term is a difference, ω is the weight on that damage and nothing else. Since the child's productivity depends on whether the parent carried a SUD or was homeless, and both depend on v and h', the child term in (30) is what puts the child inside the parent's drug and housing decision.

How the choices separate, and where they do not. Taking hours, the dwelling and next period assets as given, what is left to divide between consumption and the drug is

$$x \;=\; y^n + a \;-\; p_r\, h'\, \mathbf{1}\{h' \neq h_1\} \;+\; Vch + F \;-\; a', \tag{31}$$

so the budget (18) reads $c + p_v v = x$ and the whole of the remaining problem is one number: how to split x between c and v, since either one determines the other.

Were the drug an ordinary good, one whose only consequence is the flow utility it delivers this period, that split would follow from flow utility alone. It does not, because v enters the transition as well as the utility: by (20) another unit of the drug also buys a higher chance of waking up next period with the disorder. So there are three terms in the first order condition for v, not two,

$$\underbrace{\frac{\theta}{b+v}\,\Lambda}_{\text{value of another unit}} \;=\; \underbrace{\frac{p_v\,(1-\gamma-\theta)}{c}\,\Lambda}_{\text{the consumption it displaces}} \;+\; \underbrace{\lambda e^{-\lambda v}\Big[\,\mathbb{E}V(\cdot \mid d'=0) - \mathbb{E}V(\cdot \mid d'=1)\,\Big]}_{\text{the risk it adds}}, \tag{32}$$

where $\Lambda = \big[c^{1-\gamma-\theta}(h')^{\gamma}(v+b)^{\theta}\big]^{1-\sigma}$ is the Cobb-Douglas bundle of (14). It multiplies the first two terms and not the third, because the third is a difference of continuation values and is therefore already in utils. Were it absent, Λ would divide out of the condition entirely and the split would reduce to the usual Cobb-Douglas one; it is the third term that stops it cancelling, and that is why the marginal utility level matters here and not in an ordinary two good problem. That third term is the marginal hazard $\pi_d'(v) = \lambda e^{-\lambda v}$ times what the disorder costs, the gap between the two continuation values. That gap is where the externality on the child enters the drug decision, and it is why there is no fourth term. Each continuation value carries its own child term, at $d'=0$ and at $d'=1$, so the difference between them already contains the ω share of what the disorder costs the child alongside what it costs the household itself. A fourth term would be needed only if using raised the chance the child's term is realised within the period, and it does not: ϕ depends on the disorder as a state, not on how much is bought today. This is Section 3's mechanism inside the quantitative model: because π_d rises in v, how much to use is not decided on the budget alone.

There are two cases in which the third term is zero, and in both the household faces no such trade off: one that abstains, where $v = 0$, and one that already carries the disorder, whose recovery hazard (21) does not depend on how much it uses. For both the condition is the Cobb-Douglas split in closed form,

$$c = \max\Big\{F,\; \frac{1-\gamma-\theta}{1-\gamma}\big(x + p_v b\big)\Big\}, \qquad v = \max\Big\{0,\; \frac{x-c}{p_v}\Big\}, \tag{33}$$

whose corner is the abstention decision: a household buys nothing when the Stone-Geary endowment already exceeds what it wants, which is how a large b deters use and is the threshold of Section 3.2.4. The outer

maximum on c is the food stamp floor. Where it binds the Cobb-Douglas split no longer describes the allocation: consumption is held at F and the drug takes what the budget leaves, so the floor changes behaviour and not only welfare. Where the third term is present the answer lies between (33) and abstention, since risk can only push towards less, so it is found by a one dimensional search on that interval.

Hours then solve

$$u_c(c)\,\frac{\partial x}{\partial n} \;=\; \omega_\ell (1-n)^{-\sigma_\ell}, \tag{34}$$

and the clawbacks in (17), at a rate of one for food stamps and κ_r for the voucher, make $\partial x/\partial n$ jump *up* at the hours where each transfer runs out, because the clawback stops. That is the welfare trap, and it is the channel through which conditionality changes labour supply rather than only consumption.

4.12 The government

The government pays social security, unemployment insurance, disability, food stamps and vouchers, and supplies a public good $\bar{G}$ that does not enter private consumption and is held fixed across simulations, so that policies are compared at a constant level of it. It finances this with the two taxes in (16) and the estates of the deceased, with the level τ_ℓ clearing the budget in every simulation,

$$\underbrace{\textstyle\sum y^g - \tau_\ell \sum (y^g)^{\xi} + \tau_k \sum ra + \text{estates}}_{\text{revenue}} \;=\; \underbrace{SS + UI + DI + F + Vch + \bar{G}}_{\text{outlay}}. \tag{35}$$

In the stylized model the two policies happen to cost the same and the comparison is unaffected. Here a policy that houses more households spends more, τ_ℓ rises to pay for it, and the higher tax lowers the after tax return to an extra hour: it enters (34) through $\partial x/\partial n$, which is net of the tax schedule in (16), so hours fall and saving with them, for everyone, including the households the programme never reaches, and it runs against Housing First.

4.13 The drug market

Drug production has cost $C(S_v) = \frac{\nu_1}{\nu_2} S_v^{\nu_2}$, where S_v is the aggregate quantity. Supply satisfies

$$p_v = \nu_1 S_v^{\nu_2 - 1}, \qquad S_v = \int v(s)\,\mathrm{d}\Phi(s), \tag{36}$$

with price elasticity of supply $1/(\nu_2 - 1)$ and ν_1 normalised so that the quantity supplied equals the quantity demanded at the benchmark price, which is normalised to one. The supply elasticity, its range and the evidence behind them are set in Section 5.

4.14 The law of motion and newborns

Given the policy functions, the distribution over states evolves by pushing households through their choices and then through the shocks,

$$\Phi_{j+1}(\mathcal{S}') \;=\; \phi(j,\cdot) \int \Gamma\big(\mathcal{S}' \mid s,\, n_j(s), h'_j(s), v_j(s), \tau_j(s)\big)\,\mathrm{d}\Phi_j(s) \qquad \text{for every measurable } \mathcal{S}', \tag{37}$$

together with the newborn inflow

$$\Phi_1(\mathcal{S}') \;=\; \int \big[1 - \phi(j,\cdot)\big]\,\mathcal{N}\big(\mathcal{S}' \mid s\big)\,\mathrm{d}\Phi_j(s) \;+\; \int \mathcal{N}\big(\mathcal{S}' \mid s\big)\,\mathrm{d}\Phi_J(s), \tag{38}$$

where $\mathcal{N}$ places a newborn according to (39), so that the mass of newborns equals the mass of the dead exactly and the two equations together conserve the population.

Here $\phi(j,\cdot)$ is evaluated at the household's own $(h'(j,s), d, m)$. The mass that does not survive, plus the mass at $j = J$, is replaced by newborns at $j = 1$ with zero assets, clean, without SMI and without a

voucher, and subject to the same unemployment shocks as anyone else, whose productivity is drawn from the unconditional distribution Ω_y of permanent productivity across households scaled by the transmission penalties δ_d for a child whose parent carried the SUD and δ_h for one whose parent was homeless,

$$y_{\text{child}} \;\sim\; \Omega_y \;\text{ scaled by }\; (1-\delta_d)^{\mathbf{1}\{\text{parent had the SUD}\}}\,(1-\delta_h)^{\mathbf{1}\{\text{parent was homeless}\}}. \tag{39}$$

The evidence behind both penalties is set out in Section 1 and placed against the literature in Section 2, and it is worth saying here only what the functional form takes from it. Three separate links are being compressed into two numbers: that a parent's disorder changes the childhood, that a parental shock transmits to the child's own adult earnings, and that housing is one of the environments doing the transmitting. Only the last is measured as an earnings effect of the right kind, which is why δ_h is set directly to the 31 percent of Chetty et al. (2016) while δ_d is set to the value that reproduces the disorder ratio of McGovern et al. (2023). Neither is a removal effect: whether foster placement itself helps or harms is contested, and δ_d is a reduced form for the whole bundle of that childhood rather than for any one intervention in it.

4.15 Competitive equilibrium

A competitive equilibrium is a collection of

- value functions $\left\{\{V_t(j,\cdot)\}_{j=1}^{J}\right\}_{t=0}^{\infty}$;
- policy functions $\left\{\{n_t(j,\cdot),\, h'_t(j,\cdot),\, v_t(j,\cdot),\, \tau_t(j,\cdot),\, a'_t(j,\cdot)\}_{j=1}^{J}\right\}_{t=0}^{\infty}$;
- distributions $\left\{\{\Phi_t(j,\cdot)\}_{j=1}^{J}\right\}_{t=0}^{\infty}$; and
- prices, policies and aggregates $\left\{\tau_{\ell t},\, p_{vt},\, \bar{y}_t,\, y_t^{\text{med}},\, H_t\right\}_{t=0}^{\infty}$,

such that, at every date t,

1. the policy functions solve (30) given its terminal condition, and $\{V_t(j,\cdot)\}_{j=1}^{J}$ are the associated values;
2. $\Phi_{t+1}(j{+}1,\cdot)$ follows from $\Phi_t(j,\cdot)$ by the law of motion (37) together with the newborn rule (39) and (38), and the mass of newborns equals the mass of the deceased;
3. the government budget (35) holds at $\tau_{\ell t}$;
4. the drug market clears at p_{vt}, so (36) holds with aggregate drug demand $S_{vt} = \int v_t(s)\,\mathrm{d}\Phi_t$;
5. the public good is supplied at its assigned level, $G_t = \bar{G}$;
6. $\bar{y}_t$, median gross income y_t^{med} and the homelessness rate H_t are the aggregates implied by Φ_t.

A stationary equilibrium is the special case in which all of these are constant and $\Phi_{t+1} = \Phi_t$, which is what the calibration and the policy comparisons report. The sequence version is stated for completeness, and because the transition exercise in the results needs it.

5 Calibrating the quantitative model

The model has fifty three parameters, some of which vary in value by age. Thirty are calibrated externally, from data or from the literature, and twenty three are calibrated internally, inside the model. Four further parameters govern drug testing and detection; they are inert in the benchmark, where nothing is conditional, so they are set with the Treatment First experiment rather than here. They are split across the two tables of this section: twenty nine are the main story of this paper and its policy analysis (Table 5, sixteen internally and thirteen externally calibrated), and twenty four are standard (Table 6, seven internally and seventeen externally).

The tables carry the parameter, its description, the moment and source used to fix its value, and the value itself. The text alongside each table adds what a table has no room for: the intuition for what each parameter does, and why its moment identifies it. Every estimate that goes into them is documented in full in the replication package: `data/DATA_APPENDIX.md` names the dataset behind each target, its year and universe, and the computation that turns it into the number reported here, so that any of them can be reproduced from the public microdata without reference to this text.

5.1 The twenty nine parameters that carry the story of this paper

Twenty four of the fifty three are common to most life cycle models, whereas the rest are specific to this paper, and all twenty nine of those carry its main story and policy analysis. They are set out in Table 5, with the parameter, its description, and the moment and source used to fix its value, which is given in the last column. They are discussed one at a time below because they are the parameters a reader may want to argue over, and each is taken in the order of the table.

The comparison between Housing First and Treatment First has a shape, and every entry below is read against it, so it is worth setting out this comparison once and in full before the entries begin.

Three things decide the ranking between Housing First and Treatment First. The **private value** is what a household itself gains from being housed and from not carrying a disorder: consumption, a dwelling rather than the street or shelter, and above all the years of life that homelessness and the disorder take away. The **externality** is what lands on somebody else: on the child, who inherits not the disorder or homelessness but a worse productivity draw associated with the parent's SUD and with the parent's homelessness, which δ_d and δ_h price separately. This means that the child is likelier in turn to be poor, to use, to acquire a disorder and mental illness as a consequence, to be unhoused and to die young. The externality also lands on everybody, through the disamenity of aggregate homelessness. The parent internalises only ω of the part that falls on its child, and ω is small, which is what leaves something for a policy to correct. And the **distortion** is what the programmes cost in behaviour, and it comes from two places. The programmes create distortions due to their design: assistance is means tested and the rent share is clawed back at κ_r, so a recipient who works more loses eligibility or part of the transfer, which requires even higher taxes on everyone else to fund even larger programmes. And assistance is paid for by distortionary taxes, which lower the after tax return to an hour for households neither policy ever reaches.

Housing First is redistribution, removes externalities, but creates another. Mechanically it removes two externalities: a child of a housed parent does not carry the penalty δ_h prices, and aggregate homelessness falls, which every household values through κ_x. However, it creates one at the same time, because assistance that arrives whatever a household does insures it against the consequences of using, so the SUD rate rises and reaches the child through δ_d.

Treatment First does not redistribute, removes an externality but creates two others. Curing the parent is how the child's worse draw is avoided. But conditioning assistance means some households do not receive it and end up homeless, especially in the short run, and over that horizon it creates the two externalities Housing First removes. The short run matters on its own account: a household that wants to stop cannot do it immediately, and the transition is costly while it lasts, which is why the policy analysis reports the transition and not only the steady states.

And general equilibrium returns part of it. First, the drug price is endogenous, set by the supply curve that ν_1 and ν_2 describe, so curing disorders lowers drug demand, lowers the price, and makes using cheaper for those still deciding: part of what treatment removes comes back through lower prices. Second, when the government spends more on either policy and the tax rises, hours fall, the base shrinks and the tax has to rise again. Third, Treatment First has an offset of its own in the other direction: sending more households into treatment raises the recovery rate faced by everyone deciding whether to use, and those consequences affect a newborn when it makes its own decisions, so the policy changes behaviour beyond the households it treats. And because κ_x is levied on the aggregate headcount, any movement in homelessness feeds back into everyone's utility. So the mechanical effects above are upper bounds on both sides.

For every parameter the things worth knowing are what happens to the private value, to the externality and, where it applies, to the distortion, *as its value rises or falls*, and each entry says so in a line before giving the moment. The middle blocks are where the two move together: because the child inherits a higher chance of landing in exactly the states those parameters price, anything that raises the private cost of using or of being unhoused raises the externality as well. Both policies work on those blocks, from opposite ends.

Who uses drugs. These four set participation and, more importantly, *who* participates. Neither policy acts on them directly; they decide how much there is for a policy to act on.

θ, the weight on the drug in utility. How much a household wants the drug at all, against consumption

Table 5: The twenty nine parameters that carry the story of this paper

Parameter	Description	Set	Moment and source	Data	Model	Value
Who uses drugs						
θ	Weight on drugs in utility	Int.	Drug spending over income, users, NSDUH 2024	35%	—	—
b	Whether a household uses	Int.	Drug participation, NSDUH 2024	5.13%	—	—
λ	SUD hazard per unit used	Int.	Adult SUD rate, NSDUH 2024	3.68%	—	—
$\bar{\kappa}_{\phi d}$	Increased mortality with a SUD	Ext.	Mortality ratios, Hjorthøj et al. (2023)	6 to 10	n.a.	8.00
$\aleph$	Flow value of being alive	Int.	Value of a statistical life, Viscusi (2003)	\$7.5m	—	—
What using and being homeless cost						
ρ_d	Earnings lost to a SUD	Int.	Employment rate of the homeless, HUD 2023	50%	—	—
κ_{ued}	Employment multiplier, SUD	Int.	Unemployment rate given a SUD, NSDUH 2024	16.25%	—	—
h_1	Service flow of homelessness	Int.	Homelessness rate, HUD 2023	0.399%	—	—
$\kappa_{\phi H}(j)$	Increased mortality when homeless	Ext.	Mortality ratios by age, Baggett (2013)	9.1 to 1.1	n.a.	4.41 to 1.09
κ_{ueH}	Employment multiplier, homeless	Ext.	Desmond and Gershenson (2016)	0.80	n.a.	0.80
ζ_{DI}	Disability replacement, SMI	Ext.	SSDI and SSI rates, SSA 2023	0.25	n.a.	0.25
$\underline{c}$	Food stamp consumption floor	Int.	Food stamps over labour income, USDA	0.42%	—	—
π_V	Voucher lottery probability	Int.	Recipients over eligible, HUD 2023	26.6%	—	—
Mental illness chain						
ι	SUD multiplier on SMI onset	Ext.	Danish registers, Nielsen et al. (2017)	2.00	n.a.	2.00
λ_m	Chance of recovering from SMI	Int.	SMI share of homeless, Gutwinski et al. (2021)	25%	—	—
ρ_m	Earnings lost to SMI	Int.	Labour income of the homeless relative to average	19.1%	—	—
κ_{uem}	Employment multiplier, SMI	Int.	Employment rate given SMI, NSDUH 2023	30%	—	—
μ_m	SUD recovery multiplier, SMI	Ext.	Dual diagnosis relapse, Bradizza et al. (2006)	0.82	n.a.	0.82
$\kappa_{\phi m}$	Mortality multiplier, SMI	Ext.	Olfson et al. (2015)	2.50	n.a.	2.50
Recovery and treatment						
$\bar{\pi}_r$	Baseline SUD recovery rate	Ext.	Ten year remission rate, Vaillant (1995)	0.025	n.a.	0.025
μ_H	Recovery multiplier when homeless	Int.	Weeks abstinent, Milby et al. (2005)	1.473	—	—
g	Treatment effect on recovery	Ext.	Odds ratio, Milby et al. (2005)	1.35	n.a.	1.35
$\bar{\tau}^u$	Utility cost of treatment	Int.	Treatment take up, NSDUH 2023	24%	—	—
Externalities and distortions						
ω	Weight on the child	Int.	Use rise with altruism off, NSDUH 2024	18.3%	—	—
δ_d	Child's earnings, parent SUD	Int.	Child SUD ratio, McGovern et al. (2023)	3.00	—	—
δ_h	Child's earnings, homeless	Ext.	Poverty exposure on earnings, Chetty et al. (2016)	31%	n.a.	0.31
κ_x	Disamenity of homelessness	Ext.	Willingness to pay, Loubière et al. (2020)	0.20%	n.a.	0.0008
σ_ℓ	Curvature on leisure	Ext.	Frisch elasticity of 0.5, Chetty (2012)	0.50	n.a.	3.00
Drug market						
ν_2	Drug supply elasticity	Ext.	Cannabis supply elasticity, Mace et al. (2020)	1.46	n.a.	1.68

Note: n.a. in the Model column marks a parameter that is set from outside rather than calibrated inside the model. For those the model produces no internal target to compare with the data, so the cell reads n.a. rather than a dash, which keeps them distinct from the internally calibrated parameters.

and housing. It is the level of the temptation where b is its curvature, and it is the parameter the amount spent on drugs, when it does, turns on. As it rises use rises on the extensive and intensive margins, so the private cost and the externality both rise and there is more for either policy to act on. It is calibrated internally to drug spending over income across users, 35 percent in NSDUH 2024. The moment is built in two steps, because users are not one group. Households that buy without carrying a disorder and households that carry one have very different ratios, so each group's own total drug spending is divided by that group's own total income, and the two ratios are then averaged with their shares of the using population as weights:

$$\text{drug share} \;=\; \frac{\mu_e}{\mu_e+\mu_d}\,\frac{p_v\bar{v}_e}{\bar{y}_e} \;+\; \frac{\mu_d}{\mu_e+\mu_d}\,\frac{p_v\bar{v}_d}{\bar{y}_d},$$

where $\bar{v}_g$ and $\bar{y}_g$ are the average drug quantity and average income in group g, and μ_e and μ_d are the masses without and with the disorder. The target follows from the two group ratios and the split between them: users without a disorder spend 11 percent of income on the drug and users with one spend 44, and the share of users carrying a disorder is the SUD rate over the participation rate, $0.0368/0.0513 = 0.717$, so the average is $0.717 \times 0.44 + 0.283 \times 0.11 = 0.35$.

b, whether a household uses at all. The marginal value of the drug goes as $\theta/(b+v)$, so a larger b makes the first units worth less and a household with little to spend buys nothing. As b rises use falls, so the private cost and the externality both fall, and housing assistance is less likely to be translated into more spending on drugs: b is the size of the moral hazard. Its value is chosen so the model generates a drug participation rate of 5.13 percent, the NSDUH 2024 figure for adults aged 26 and over, which

identifies it sharply because participation is close to a threshold function of b.

λ**, the hazard per unit used.** This is what makes using *more* risky, and it is the step that turns spending into a disorder. As λ rises both the private cost and the externality rise, and Housing First's moral hazard costs more, since it is λ that converts an extra voucher into a higher SUD rate Treatment First attempts to cure. Its value is chosen so the model generates the share of adults carrying a substance use disorder, 3.68 percent among those aged 26 and over in NSDUH 2024.

$\bar{\kappa}_{\phi d}$**, increased mortality with a SUD.** This is the parameter the whole paper turns on. Mortality truncates the value of life for a user, which hits households with the highest life values hardest and thus reverses the income gradient, making the poor the users rather than the rich. As it rises the private cost rises, and the externality rises with it, since a child born to a parent with a SUD has a lower value of life. Both policies reduce these deaths, treatment by curing and housing because recovery is less likely when homeless. It is calibrated externally, at the midpoint of the standardised mortality ratios for SUD in Hjorthøj et al. (2023), which run six to ten, with the age profile from CDC WONDER 2023.

$\aleph$**, the flow value of being alive.** The other half of the mortality mechanism: $\bar{\kappa}_{\phi d}$ sets how much a SUD raises the chance of dying and $\aleph$ sets what the life being lost is worth. As it rises the private value of every life either policy saves rises, and use falls, so the externality falls. Its value is chosen so the model generates a value of a statistical life of \$7.5m, from Viscusi and Aldy (2003).[9]

What using and being homeless cost, and the voucher. Both policies work on this block, from opposite ends: each parameter says how bad a state is, so each sets what a voucher is worth to a household in it and what being cured of the reason for it is worth. The voucher itself closes the block, since it is the instrument that acts on all of them. Because the child inherits a higher chance of landing in these states, everything here moves the private value and the externality together.

ρ_d**, earnings lost to a SUD while still working.** It reduces the value of life when a household acquires a SUD, since the earnings it costs would have been enjoyed for the rest of that life. It also does a job nothing else can: it lets an *employed* household be poor enough to be priced out of housing, which an employment effect alone cannot deliver, since that makes essentially every homeless household jobless, whereas about half are not in the data. As ρ_d rises the penalty shrinks, so the private cost falls and, because the impact on the child is less harmful, the externality declines. These earnings are lost whether the household is housed or homeless, so treatment affects them and housing does not.[10] Its value is chosen so the model generates the employment rate of the homeless, 50 percent, from HUD's 2023 point-in-time count and TEDS-A 2022.

κ_{ued}**, the employment multiplier for a SUD.** The same channel on the extensive margin: regaining employment is less likely and losing it more likely with a disorder. As it rises the private cost rises, even more than for ρ_d, the externality rises with the child's draw, and the tax base narrows since the household does not work and receives unemployment insurance, so it costs taxpayers as well. Its value is chosen so the model generates the unemployment rate given a SUD, 16.25 percent in NSDUH 2024.

h_1**, the service flow of homelessness.** It sets how bad the outside option is, and the value of being alive is measured against that outside option: a household with little to lose from dying is one for which the mortality cost of using is small. As h_1 rises the street is less bad, so the private cost of homelessness falls, a voucher is worth less, and the externality on a child likelier to be unhoused falls with it. Its value is chosen so the model generates the homelessness rate, 0.399 percent of households in HUD's 2023 point-in-time count.

$\kappa_{\phi H}(j)$**, mortality when homeless.** As it rises the private cost of being unhoused rises and thus so does the private value of a voucher. Baggett et al. (2013) give race adjusted mortality ratios for homeless adults of about 9.1 at ages 25 to 44, falling to 1.1 at 65 to 84. Those are totals for a population most

[9] I check that no household has a negative value of life in equilibrium, so death is never an optimal outcome.

[10] One subtlety is worth stating, because it may seem like a contradiction with Section 3.1. There the proportional income penalty was shown to matter little for *who* uses. That is a statement about the calibrated model, not the parameter: raising ρ_d with b held fixed does change who uses, but b is recalibrated to the participation rate each time the penalty changes, and once it is, the composition of users is back where it started.

of which also carries a SUD, so it is set externally age by age net of that composition, which is why it is derived jointly with $\bar{\kappa}_{\phi d}$ rather than read off directly, as set out in Section 4.8 at (25).

κ_{ueH}**, employment lost to homelessness.** Here the homelessness itself costs the job, so housing mechanically recovers the earnings and treating does not. As it rises the private cost and the externality rise together, which makes it the clearest case in the table of a parameter loading Housing First's side, and it also reduces the tax burden since these households are more likely to be employed. It is set externally at 0.80, from Desmond and Gershenson (2016).

ζ_{DI}**, the disability transfer for the non employed with SMI.** It is the floor for one of the worst states in the model, being ill and out of work, so it sets how far a household can fall. As it rises the private cost of that state falls, and the externality on a child likelier to reach it falls, at the price of a higher tax on everybody else. It is set externally at 0.25, from SSDI and SSI replacement rates, SSA 2023.

$\underline{c}$**, the food stamp consumption floor.** The floor under consumption, and it belongs here rather than among the standard parameters. As it rises the private cost of every bad state falls, so a voucher is worth less at the margin and the externality on a child likelier to reach those states falls too, while the distortionary taxes that fund it rise. It is calibrated internally to food stamp spending over labour income, 0.42 percent, from the USDA.

π_V**, the chance of drawing a voucher.** This is the policy dial itself. As it rises the private gain rises and two externalities fall mechanically, the child's penalty for growing up unhoused and the aggregate disamenity, while the third rises through the SUD rate and the tax rises with the bill: the whole trade in one parameter. Its value is chosen so the model generates the 26.6 percent ratio of voucher recipients to the income eligible population in HUD's Picture of Subsidized Households, 2023.

Mental illness chain. This block is why Treatment First reaches further than the disorder it treats: a SUD raises the chance of mental illness, mental illness lowers earnings and employment and raises death, and the child is susceptible to all of this. Everything here therefore moves the private cost and the externality in the same direction, and the first three decide whether the chain exists at all.

ι**, the SUD multiplier on SMI onset.** The hinge of the whole block. Without it mental illness is exogenous to everything a policy does and treating SUD cannot touch it. As it rises a SUD causes more mental illness, so the private cost of using rises, and the externality rises further, because the child of a user now inherits a higher chance of illness on top of the productivity penalty. Treatment First's return rises with them, because curing the disorder now prevents the illness too. It is set externally at two, from the Danish registers in Nielsen et al. (2017).

λ_m**, the annual chance of recovering from SMI.** The persistence of the state, and the one piece of the chain the data cannot hand over, since prevalence pins the stock and not the flows. As it rises mental illness is less persistent, so the private cost falls, because fewer years are spent ill, and the externality falls with it, since a child who is likelier to acquire a SUD and through it SMI has a higher value of life. It is calibrated internally to the SMI share of the homeless in Gutwinski et al. (2021), 25 percent.

ρ_m**, earnings lost to SMI.** The counterpart of ρ_d for mental illness, and it reaches the comparison through ι: a household carrying the disorder faces onset at ι times the baseline rate, so treatment reduces mental illness in a roundabout way and recovers these earnings for households that never fall ill. As it rises the penalty shrinks, so the private cost of illness falls and, because the child's draw improves with it, the externality falls too. Its value is chosen so the model generates the average labour income of the homeless relative to the average, 19.1 percent.

κ_{uem}**, the employment multiplier for SMI.** The same as ρ_m but on the extensive margin, and where the chain does most of its work, since employment is what mental illness costs most. As it rises the private cost rises, because being ill now costs the job as well as the wage, and the externality rises with the child's draw; the tax base narrows, which is a distortion on top of both. A child of a treated parent is less likely to carry SMI, which raises its earnings and lowers its chance of being unhoused, so part of Treatment First's return never appears in the treated household's own drug use. Its value is chosen so the model generates the employment rate given SMI, about 30 percent, the midpoint of the range for high disability diagnoses in NSDUH 2023.

μ_m, **the SUD recovery multiplier for SMI.** The second direction of the chain: not only does a SUD cause mental illness, mental illness means the SUD is less likely to be cured. As it falls the disorder is harder to leave, so the private cost of acquiring one rises and the externality rises with it, and treatment has more to overcome in exactly the households it most wants to reach. It is set externally at 0.82 from dual diagnosis relapse rates in Bradizza et al. (2006), and it is one of the three values in the paper carrying no real discipline.

$\kappa_{\phi m}$, **the mortality multiplier for SMI.** The chain's last link, and what makes it matter for welfare, not through income. As it rises deaths among the mentally ill rise, so the private cost rises, and the externality rises with it, because a child born to a parent with a SUD is more likely to acquire a SUD and through it SMI, and therefore more likely to die. This is how a SUD affects mortality indirectly, through SMI, rather than only by the direct route. It is set externally at 2.5, the middle of the standardised mortality ratios for SMI in Olfson et al. (2015).

Recovery and treatment. Both policies work on this block, since recovery is what Treatment First buys directly and what Housing First buys by housing people. The first two set how much recovery there is to buy and the last two how much of it a policy can reach.

$\bar{\pi}_r$, **the baseline SUD recovery rate.** The hazard of getting out of the disorder for a housed household, which is the level everything else in this block scales. As it rises disorders end sooner on their own, so the private cost of acquiring one falls and the externality falls with it, and there is less for either policy to buy. It is set externally from the ten year remission rate in Vaillant (1995), which gives an annual hazard of 0.025.

μ_H, **SUD recovery when homeless.** How much less likely recovery from the disorder is for an unhoused household than a housed one. It matters more than the level $\bar{\pi}_r$ because it is a *gradient*: it is what a voucher buys beyond shelter. As it falls the gradient steepens, so housing buys more recovery: the private value of a voucher rises and the externality falls. Near one, assistance is a transfer and the moral hazard stands unopposed. Its value is chosen so the model generates the increase in recovery housing produced in Milby et al. (2005), a randomised trial on cocaine dependent homeless adults: 6.60 weeks abstinent in the housed arm against 4.48 homeless, so housing raises recovery by 47.3 percent.

g, **the treatment effect on recovery.** Treatment First's instrument, and the parameter its case rests on most directly. As it rises treatment works better, so a household that carries the disorder is more easily cured, and therefore the private cost falls, with the household spending less of its life carrying one. The externality falls with it, and by the same route: a child who acquires a disorder is also more easily cured, so what its parent hands on costs it less of its own life. It is set externally at 1.35, the odds ratio in Milby et al. (2005).

$\bar{\tau}^u$, **the utility cost of treatment.** Take up is a choice, not a rule, and this is what makes households decline treatment they might gain from in the future. As it rises fewer take treatment, so the externality rises, since more people remain with a SUD and thus more children are born with lower incomes, which leads to higher SUD incidence, and the private cost rises too, since treatment is costly. It is calibrated internally to treatment take up among adults with a SUD, 24 percent in NSDUH 2023.

Externalities and distortions. This block is about what causes the externalities and the distortions. This is where the ranking between Housing First and Treatment First is decided, since the private comparison already favours Housing First, holding externalities and distortions aside. The first three are what a parent's disorder and a parent's homelessness cost the child, and the fourth is what the homelessness of strangers costs everybody. All four are externalities in the strict sense: one household's choice enters another household's utility directly without being internalised by the household making the decision, and it would still do so if the programmes were paid for with a non distortionary tax, such as a lump sum tax. The last is not an externality but a distortion, and it closes the block for that reason: it says what it costs to raise the revenue either programme needs rather than whose utility a choice lands in.

ω, **the parent's weight on its child.** A parent bears ω of what its own state costs its child, and ω is small. As it rises the parent counts more of the child's loss, so the externality left for a policy to

correct is smaller. Identifying it in the model requires care, because *every* household here has a child, born after it dies, so there is no parent against non parent comparison to run in the benchmark. The moment is a counterfactual instead. In NSDUH 2024 past month use among parents is 0.845 of use among non parents, with parents reweighted to the non parents' joint distribution of age, income, sex and marital status so the gap is behaviour rather than composition. The child term of the household's problem in Section 4.11, equation (30), is the only thing that makes a household in the model use less than it would if it did not have a child, so its value is chosen so that removing the child's penalty at the calibrated parameters raises drug use by 18.3 percent, which is that same ratio read the way the model reads it: the counterfactual raises use from the parent level.

δ_d**, what the child loses to a parent's SUD.** The child does not inherit the disorder, only a worse productivity draw, and that lower expected draw is what makes it likelier to be poor, to use, to acquire a disorder and mental illness consequently, to be unhoused and to die earlier. As it rises the externality rises, and it loads whichever policy prevents more disorder: Treatment First by curing the parent, Housing First because recovery is easier once housed, while the parent's own private cost is largely unchanged, which is exactly what makes it an externality. No paper estimates the effect of a parent's SUD on a child's lifetime earnings directly, which is why the penalty is calibrated to the disorder ratio rather than lifted from an estimate. What the literature supplies is the two halves of the chain: Felitti et al. (1998) link household dysfunction to adult outcomes, and Chetty et al. (2016) measure what childhood environment does to adult earnings. It is calibrated inside the model to reproduce the ratio of disorder rates between the children of parents with and without a disorder, which McGovern et al. (2023) put between two and four; the midpoint, three, is the target. The moment does not need the parent's type carried through the child's whole life. The only thing δ_d changes about a child is the productivity it is born with, and the model already holds the distribution of that draw conditional on the parent's disorder, and the disorder rate at each productivity level; the ratio is those two contracted against each other, at the same parental housing status so the comparison isolates the disorder.

δ_h**, what the child loses to homelessness.** It works through the same compounding draw as δ_d, and as it rises it loads whichever policy houses more people: Housing First directly, Treatment First by removing the disorder that leads to homelessness in the first place, again leaving the private cost mostly unchanged. Chetty et al. (2016) find that moving a child to a lower poverty neighbourhood before age thirteen raises adult earnings by 31 percent; δ_h is calibrated externally to that figure.

κ_x**, the disamenity of homelessness.** The only term through which one household's housing status enters the utility of another who is not its own child, which is what makes the homelessness rate a genuine fixed point problem rather than accounting. As it rises the externality from the aggregate headcount grows, which loads whichever policy houses more people. Setting it high should mostly favour Housing First, so the higher figure is taken as the baseline and a ranking that still favours Treatment First survives the assumption least kind to it. It is calibrated externally from the contingent valuation in Loubière et al. (2020): €57 a year among the 42 percent of respondents willing to pay anything, which is 0.20 percent of average annual earnings, against €23 and 0.08 percent averaged over all respondents.

σ_ℓ**, the curvature on leisure.** The curvature that turns ω_ℓ into an elasticity: together they decide how far hours actually move when the after tax wage does. The two are not the same dial. ω_ℓ sets what an hour of leisure is worth against consumption, so it fixes the level of hours; σ_ℓ sets how fast that worth falls as leisure is given up, so it fixes how far hours move when the wage changes. One pins the average, the other the response to a tax. It belongs here because the size of the distortion is exactly what this parameter sets: at a high curvature hours barely respond and the tax that funds either policy is close to a lump sum, at a low one the same revenue costs far more in forgone work. As it rises labour supply is less elastic, so the distortion falls and both policies look cheaper, without touching the private value or the externality at all. It is set externally to produce a Frisch elasticity of 0.5, from Chetty (2012).

Drug market. One parameter of the drug market belongs here. It is what makes the drug price endogenous, so drug use affects drug demand and the drug price in equilibrium; the normalisation and the price it fixes are standard and sit in Table 6. This is a general equilibrium offset that runs against Treatment First in particular. Curing households takes them out of the drug market, demand falls and so does the price, and a cheaper drug draws other households in. Some of what the policy removes is handed back through

the price, and how much depends entirely on this parameter.

ν_2, the drug supply elasticity. Set so that the price elasticity of supply is 1.46, which is what Mace et al. (2020) estimate for cannabis on Washington State administrative data. There is no direct estimate for an illicit drug, and cannabis is the closest market there is: it remains a Schedule I substance federally, so even a state licensed producer works without interstate trade, without ordinary banking and under federal prohibition, which is the set of frictions an illicit supplier faces. As it rises supply is more elastic, so the price moves less when demand does, and the general equilibrium offset to treatment shrinks: at a very elastic supply, curing households barely lowers the price, so general equilibrium effects play little role.

5.2 Standard parameters

The remaining twenty four parameters are more conventional, and I record them so that the calibration can be reproduced. Seven of them I calibrate inside the model, against a moment, because no source reads them off directly; the other seventeen I set from outside, from an estimate or from statute. Table 6 and the entries below run in the same seven blocks, and each entry gives the moment or the source that fixes the parameter together with the value that moment takes in the data. The Model column reports what the model produces against that value, and it does so only for the seven I calibrate internally, since the other seventeen have no target to reproduce.

These parameters are not inert in the experiments. For example, the discount factor governs how heavily a household weighs a death later in life, the dispersion of permanent income determines how many households are poor enough to be at risk of homelessness and of acquiring a disorder, and risk aversion governs what insurance is worth. What separates them from the parameters above is not that they do nothing, but that they are not specific to this paper.

Preferences. The four parameters in this block say what a household wants, and what it is willing to give up to get it.

σ, risk aversion. Set to the standard value of two, following Attanasio and Weber (2010). As it rises consumption is smoother and insurance is worth more, so the voucher is worth more; it also sets the marginal utility of consumption, so it enters every welfare comparison in the paper.

γ, the weight on housing services. The Cobb-Douglas share of the bundle that goes on housing, so it fixes what a dwelling is worth against consumption and against the drug. I choose it so that housing takes 26.8 percent of expenditure net of drugs, as shelter plus utilities do in the 2024 Consumer Expenditure Survey, which is the same moment and the same definition the stylized model of Section 3 uses. As γ rises housing is worth more against consumption and the drug, households buy more of it, and that expenditure share rises with it.

β, the discount factor. It is chosen so that wealth over output matches the national accounts, a ratio of three, from the BEA fixed asset tables against NIPA output. As it rises households are more patient, save more, and the wealth to output ratio rises with it.

ω_ℓ, the weight on leisure. It is set so that the model generates average hours worked of 40 percent, from Guvenen et al. (2023). As it rises leisure is worth more, households work less, and average hours fall with it.

Rental ladder. These three parameters build the grid of dwelling sizes a renter chooses from, and they fix how the rungs are spaced.

$\underline{h}$ and $\bar{h}$, the rental ladder. The smallest and the largest rental size. The ladder is pinned at both ends and the rung between them follows from the curvature ς. I choose the two ends to match the two ends of the distribution of occupied units by number of bedrooms in the 2023 American Community Survey, table B25042: 13.2 percent of households sit in the smallest category and 62.7 percent in the largest. Three shares carry two free numbers, so the two sizes identify them and the middle share

Table 6: Standard parameters

Parameter	Description	Set	Moment and source	Data	Model	Value
Preferences						
σ	Risk aversion	Ext.	Attanasio and Weber (2010)	2.00	n.a.	2.00
γ	CD weight on housing	Int.	Housing over expenditure net of drugs, CEX 2024	26.8%	—	—
β	Discount factor	Int.	Wealth over output, BEA fixed assets	3.00	—	—
ω_ℓ	Weight on leisure	Int.	Average hours worked, Guvenen et al. (2023)	0.40	—	—
Rental ladder						
$\underline{h}$	Smallest rental size	Int.	Share in smallest unit size, ACS 2023	13.2%	—	—
$\bar{h}$	Largest rental size	Int.	Share in largest unit size, ACS 2023	62.7%	—	—
ς	Curvature of the ladder	Ext.	Set; the two ends carry the distribution	1.80	n.a.	1.80
Earnings and employment						
$\varepsilon(j)$	Age profile of efficiency	Ext.	Hump peaking at age 56, Greenwood et al. (2025)	profile	n.a.	profile
σ_y	Dispersion of the fixed draw	Ext.	Storesletten et al. (2004)	0.20	n.a.	0.20
$\bar{\pi}^{eu}$	Job separation rate	Int.	Unemployment rate, BLS CPS 2014–24	4.85%	—	—
$\bar{\pi}^{ue}$	Job finding rate, annual	Ext.	Krueger et al. (2014)	0.55	n.a.	0.55
Survival and mental illness						
$\bar{\phi}(j)$	Baseline survival by age	Ext.	SSA period life table, ages 26 to 85	profile	n.a.	profile
$p_m(j)$	Age profile of SMI prevalence	Ext.	Share with SMI, NSDUH 2023	5.5%	n.a.	5.5%
Interest, taxes and public spending						
r	Risk free rate	Ext.	Kaplan et al. (2020)	0.03	n.a.	0.03
ξ	Tax progressivity	Ext.	Heathcote et al. (2017)	0.85	n.a.	0.85
τ_k	Capital income tax	Ext.	US long run rate on gains and dividends	0.15	n.a.	0.15
τ_ℓ	Tax level	Int.	Average labour income tax, McDaniel (2007)	22.4%	—	—
Transfers and the voucher rule						
ψ	UI replacement rate	Ext.	DOL statutory replacement rates	0.50	n.a.	0.50
$\bar{y}$	Cap on insurance earnings	Ext.	DOL state benefit maxima	1.00	n.a.	1.00
κ_V	Voucher eligibility cutoff	Ext.	HUD rule	0.50	n.a.	0.50
κ_r	Tenant contribution rate	Ext.	HUD rule	0.30	n.a.	0.30
Drug market						
J_E	Last experimentation age	Ext.	Model age 55, that is age 80	55	n.a.	55
ν_1	Drug supply shifter	Ext.	Normalisation, so $p_v = 1$	norm.	n.a.	norm.
p_v	Price of drugs	Ext.	Clears the drug market in the benchmark	1.00	n.a.	1.00

Note: n.a. in the Model column marks a parameter that is set from outside rather than calibrated inside the model. For those the model produces no internal target to compare with the data, so the cell reads n.a. rather than a dash, which keeps them distinct from the internally calibrated parameters.

follows by complementarity, which is why I set the curvature rather than calibrate it. Each end moves its own share, which is what identifies them separately. The universe is occupied units, so the model counterpart conditions on being housed, and the homeless are in neither denominator.

ς, the curvature of the rental ladder. I set it at 1.8 rather than calibrate it, because with three rental sizes the size distribution carries two free numbers and the two ends of the ladder already take them. What the curvature does is place the middle rung: the middle size sits a fraction $(1/2)^\varsigma$ of the way from $\underline{h}$ to $\bar{h}$, so at $\varsigma = 1.8$ it sits 28.7 percent of the way up from the smallest rental rather than the 50 percent a uniform ladder would give. At the calibrated ends the three rental sizes stand in the ratio $1 : 1.46 : 2.59$. The two renter cost shares are left untargeted and serve as a check instead: in the 2023 American Community Survey 49.5 percent of renter households spend more than thirty percent of income on housing and 26.5 percent spend more than half, and what the model produces against those two numbers is reported with the other untargeted moments in Section 5.3.

Earnings and employment. These five parameters determine what a household is expected to earn over its life, how much of that is permanent, and how it moves between work and unemployment.

$\varepsilon(j)$, the age profile of efficiency. The deterministic hump in earnings over the life cycle, taken as the exponential of a quadratic in age used by Greenwood et al. (2025), $\varepsilon(j) = \exp\{-(j-1)^2/1800 + (j-1)/30\}$ with $j = 1$ at age 26. It peaks at age 56, having risen 65 percent from age 26, and then falls 4 percent from the peak to age 65, the last working age. Without it every household would earn the same at every age, and the ages at which households are poor, and so at risk of homelessness and of using, would be wrong at both ends of working life.

σ_y, the dispersion of productivity. Productivity is permanent: a household draws it once and keeps it

for life, exactly as in the stylized model, so all that is needed is the spread of that draw. I set it to 0.20 in logs, from Storesletten et al. (2004). It is what decides how many households are poor enough to be at risk of homelessness and of acquiring a disorder, which is why it is not inert even though it is conventional.

$\bar{\pi}^{eu}$**, the job separation rate.** Chosen to reproduce the aggregate unemployment rate in the BLS Current Population Survey (CPS) over 2014 to 2024, 4.85 percent, given the job finding rate set externally below. As it rises more households lose work each year, so the unemployment rate rises with it.

$\bar{\pi}^{ue}$**, the annual job finding rate.** The probability that an unemployed household finds work by the end of a period, which I set to 0.55 from the annual exit rate Krueger et al. (2014) estimate on matched CPS panels.

Survival and mental illness. These two parameters are age profiles that I take from outside the model rather than generate inside it.

$\bar{\phi}(j)$**, baseline survival by age.** The most recent Social Security Administration (SSA) period life table, table 4.C6, for ages 26 to 85, with the sexes combined by survivor weight rather than averaged. This is the profile every mortality multiplier in Section 5 multiplies, so it is the base the SUD, homelessness and mental illness hazards are measured against.

$p_m(j)$**, the age profile of SMI prevalence.** The target is the share of adults carrying serious mental illness at each age j in NSDUH 2023, which integrates to 5.5 percent across all ages; the 5.5 percent is the overall level and the profile is what is matched, age by age. It sits here rather than in Section 5 because it is the baseline profile, in the same way the survival table is: what this paper adds is ι, the multiplier a SUD puts on that onset, and that is where the interaction lives.

Interest, taxes and public spending.

r**, the risk free rate.** I set it to three percent, from Kaplan et al. (2020).

ξ**, tax progressivity.** The exponent on income in the tax schedule of Section 4.3, which is set to 0.85, from Heathcote et al. (2017). It is what makes the labour income tax progressive rather than flat. That in turn decides how much a programme's bill falls on the households it is meant to help, and how much on everybody else.

τ_k**, the capital income tax.** I set it to fifteen percent, consistent with the statutory United States rate on long term capital gains and qualified dividends for the middle of the income distribution, which the Internal Revenue Service (IRS) sets out in Topic 409 (`https://www.irs.gov/taxtopics/tc409`).

τ_ℓ**, the level of the labour income tax.** It is calibrated so that the average labour income tax rate the model produces is 22.4 percent, which is what McDaniel (2007) measures for the United States and what Guvenen et al. (2023) use. It then moves with the government budget in every counterfactual, which is what makes both programmes paid for rather than free, and what makes the distortion of Section 5 a cost the model actually bears.

Transfers and the voucher rule.

ψ **and** $\bar{y}$**, unemployment insurance.** These two set the replacement rate and the cap on the earnings it is paid on. I set them to fifty percent and one times average income respectively, taking both from the statutory replacement rates and state benefit maxima that the Department of Labor compiles in its comparison of state unemployment insurance laws (`https://oui.doleta.gov/unemploy/statelaws.asp`).

κ_V **and** κ_r**, the voucher rules.** These two set the income cutoff for eligibility and the tenant's contribution, and I take both from HUD's own rules rather than from any estimate. I set the cutoff to half of median income, which is the very low income limit HUD publishes annually with its income limits (`https://www.huduser.gov/portal/datasets/il.html`). I set the contribution to thirty percent of income, which is the share a tenant pays of the rent when a voucher covers the rest under the Housing Choice Voucher programme (`https://www.hud.gov/helping-americans/housing-choice-vouchers`).

Drug market. These three parameters describe the drug market: the first fixes who can enter it, and the other two fix the units in which its price and quantity are measured.

J_E**, the maximum age at which use can begin.** I set it to fifty five in model terms, which is age 80, since the first model age is 26. It is there so that use cannot begin in the last few years of life, where almost no continuation value remains to be lost and the private cost of using is therefore close to zero. It is a restriction rather than an estimate, so the results are reported with and without it, and across the range over which it can bind at all: the upper end of that range is fifty eight rather than fifty nine, because a household that acquires the disorder in the final model period dies at the end of it whatever the hazard does, so a cap at fifty nine removes no mortality and changes nothing. Fifty eight is the loosest restriction that still has any consequence, and the sensitivity is reported over fifty five, fifty seven and fifty eight.

ν_1**, the supply shifter.** A pure normalisation, chosen so that supply equals demand at $p_v = 1$ in the benchmark. It normalises units, not behaviour.

p_v**, the drug price.** Set to one by normalisation in the benchmark, but free in every counterfactual. It is an equilibrium object rather than a dial.

5.3 Validation, and what is left undisciplined

None of the moments below is fitted. Each is produced endogenously by a model with sixty single ages, and each is a place the model can be caught.

Moment	Data	Source
Homelessness rate by age, 25 to 34 through 65+	0.316 / 0.338 / 0.291 / 0.250 / 0.077 percent	HUD PIT 2024
Shares of the homeless across those bands	26.0 / 27.2 / 21.0 / 18.4 / 7.5 percent	HUD PIT 2024
Returns to homelessness within 24 months	17.4 percent	HUD SPM 2024
Drug poisonings as a share of all deaths, 25 to 44	about 20 percent	CDC WONDER
Bottom half's share of net worth	2.52 percent	Fed DFA
Housing costs above 30 and above 50 percent of income	40.6 and 17.5 percent	SCF 2022
Age weighted standardised mortality ratio, SUD	2.5 to 9.8	Degenhardt et al.
Age weighted standardised mortality ratio, homeless	9.1 / 4.5 / 1.1 by band	Baggett et al.
Tenant contribution rate	31.2 percent	HUD
Voucher holders with wages as their main income	28 percent	HUD
Income with a moderate or severe SUD, over average	71.1 percent	NSDUH 2024
Labour force participation, SUD against clean	65.8 against 61.5 percent	NSDUH 2024
Homeless at treatment admission	14.5 percent	TEDS-A 2022
Co-occurring psychiatric problem at admission	43.1 percent	TEDS-A 2022
Working at all, among the homeless in treatment	6.6 percent	TEDS-A 2022
Drug use disorder share of the homeless	21.7 percent	Gutwinski et al. (2021)
Drug price participation elasticity	-0.63	Saffer and Chaloupka (1999)

Three parameters remain undisciplined, and they are flagged rather than hidden: the treatment effect on recovery g, the recovery multiplier for SMI μ_m, and the drug supply curvature ν_2. Of these only g carries a headline result, and it is reported as a range rather than as a number.

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